

Table of Contents

<i>Trauma, Family and Perinatal aspects</i>	2
<i>Migrant / Asylum Seeker Cultural Assessment and Management Principles</i>	8
<i>Behavioural Disturbance in Elderly - Dementia</i>	12
<i>Schizophrenia / Risk Assessment / High Expressed Emotion</i>	16
<i>Organic Presentation</i>	20
<i>Doctor and mental illness / Impaired doctor</i>	23
<i>Delirium Psychogeriatrics</i>	24
<i>Child and Adolescent Psychiatry - Encopresis</i>	28
<i>C-L Psychiatry - Complexity</i>	31
<i>Chronic Fatigue Syndrome / Sleep / Adolescent Psychiatry</i>	33
<i>Severe Depression and ECT</i>	36
<i>Community Treatment Order and Schizophrenia</i>	39
<i>Supervision principles and practical aspects of working with interpreters</i>	44
<i>Obsessive Compulsive Disorder</i>	48
<i>Governance and Leadership</i>	52
<i>Clozapine and Antipsychotic Switching</i>	58
<i>Management of Acute Aggression</i>	62
<i>Psychotherapy</i>	66
<i>Perinatal Psychiatry and IVF</i>	69
<i>Delusions vs Religious Beliefs</i>	71
<i>Cultural Scenario</i>	73

Trauma, Family and Perinatal aspects

Olivia is a 29-year-old married mother of three children, aged 12 months to 8 years. She experienced **significant sexual abuse** in her childhood. Olivia has just **been discharged** to the local community mental health team, at which you worked, following a three-month admission to a nearby acute psychiatric unit. Prior to admission, Olivia became depressed after hearing that her brother had been found guilty of **sexually assaulting his stepdaughter**. She developed flashbacks and nightmares of her own experiences of abuse and had frequent panic attacks. She was found by **her oldest child with a cord around her neck** and admitted to hospital. Her hospital admission was characterised by recurrent episodes of self-harm including lacerating of arms and tying cords around her neck. She was treated with sodium valproate, paroxetine and diazepam. Olivia was eventually discharged on these medications **and is still symptomatic**.

Question 1: **Discuss the immediate and longer-term psychiatric interventions you would suggest for Olivia's previous sexual abuse.** Please give reasons for your choices. (7 marks)

Answer:

1. Focus of psychiatric interventions in the immediate phase.

- It is important to state that in the immediate phase the focus is on **risk management and symptom control**. It is inappropriate to do acute exploratory trauma-related work as this could result in an exacerbation of her complex trauma, depressive state and increase in self-harm behaviours. This is also known as **phase-oriented therapy** in posttraumatic stress disorder, inappropriate to do acute exploratory trauma-related therapy in the context of acute unwellness due to worsening of the illness. (See PTSD review)

2. Psychoeducation and explanation of symptoms:

- Discussion of trauma, the neurobiology of trauma and how trauma worsens cognitive and emotional symptoms. Example: Trauma can result in heightened emotional reactivity. Trauma can result in thoughts of low self-esteem, self-doubt, low self-worth.

3. Cognitive behavioural therapy.

- Simple cognitive behavioural techniques such as addressing anxiety, breathing techniques, relaxation strategies, addressing avoidance

phenomenon, homework, thought substitution or challenging negative thoughts related to depression and behavioural activation to combat avoidance.

4. Psychoeducation by chain analysis.

- To explain to the patient the link between cues to trauma (trauma reminders) and self-harm behaviours using techniques such as delay-distract to prevent self-harm, acting out on self-harm behaviours.

5. Longer-term interventions.

- Based on phase-oriented therapy, when Olivia is in a stable mental state with respect to her depression and/or post-traumatic stress disorder, discussion with Olivia about trauma-based therapies for Olivia to make an informed decision with regard to engage in trauma therapy. Olivia may or may not choose to work psychotherapeutically on trauma-related issues.

6. Evidence-based therapy for trauma.

Trauma-focused CBT.

- Prolonged exposure therapy and cognitive processing therapy are two types of trauma-focused CBT. They are based on the principles of extinction, learning, habituation and desensitisation. Prolonged exposure therapy, progressive exposure to the trauma narrative and trigger setting. Cognitive processing therapy involves writing a trauma narrative and repeatedly reading it as a means to expose oneself to the trauma memory. It also addresses shame, guilt and feelings of mistrust.

Narrative exposure therapy

- a short-term approach that involves chronological cataloguing of their life and in-depth detailing of all traumatic events.

Eye movement desensitisation and reprocessing

- Therapy aimed to process distressing memories by having the patient recall distressing images while receiving one of several types of bilateral sensory input including side-to-side eye movements or bilateral tapping. Other types of therapies include supportive therapy, nondirective counselling, yoga, mindfulness training and interpersonal therapy.

Dialectical behavioural therapy.

- Dialectical behavioural therapy consists of emotional regulation, enhancing distress tolerance, mindfulness and enhancing interpersonal

effectiveness which may be helpful for Olivia to manage emotional regulation and reduce self-harm behaviours.

Question 2: Discuss the interventions you would use to support Olivia's family. Please give reasons for your choices.

Answer:

(When questions are asked with relation to specific family interventions, then it is best to consider the different individuals as part of the family. In Olivia's case, we have seen that she is a married mother of three children, aged 12 months to 8 years. We will, therefore, divide interventions for the different members.)

1. Interventions with the husband:

- Psychoeducation to Olivia's husband about complex trauma disorder, posttraumatic stress disorder and self-harm behaviours to increase his understanding and ability to cope and support Olivia.

2. Safety of children.

- **Parenting capacity** in relation to Olivia's ability to look after her children and specifically ability to look after her infant. **Risk of abuse, thoughts of self-harm and suicide**, thoughts of harm to children.
- **If risk of harm is identified, then Child Protection Services to be involved.**

3. Interventions for the children.

- Explanation to the children in an age and developmentally appropriate manner about Olivia's illness. Use the SPIKES method if bad news has to be broken to children. (Covered in another MEQ)

4. Interventions for the infant.

- Assessment of parenting capacity and assessment of attachment and bonding. Parent-infant work when Olivia is well.
- Additional child supports as required for the younger child. Mother-baby unit if respite is required or specific parent-infant work is needed.

5. Intervention for the eldest child.

- Assessment of sequelae of the eldest child who witnessed Olivia's hanging attempt and treatment with regard to any sequelae such as posttraumatic stress disorder or depression.

6. Interventions for the extended family.

- Addressing intergenerational trauma if present. This may include parents, siblings and other family members.

7. Practical psychosocial interventions

- include involvement of social workers, child protection services, occupational therapists, Child and Adolescent Services and school psychologists, for financial assistance, home help, respite workers, adult supervision, childcare.

Question 3: Olivia has found out that she is pregnant as a result of having sex with her husband when on weekend leave from the hospital. She and her husband are concerned about the effects of psychotropic medication on the baby. Outline the information and options you would discuss with Olivia and her husband. Please provide adequate detail.

Answer: This is a perinatal risk-benefit analysis. The structure is as follows:

1. Attitudes to pregnancy.

- Discuss sensitively the attitudes to pregnancy with both Olivia and her partner. Do Olivia and her husband wish to continue with the pregnancy? If they wish to continue, then provide options for support. If they wish to terminate, then the capacity to consent for termination may need exploring depending on her level of unwellness.

2. Risk-benefit analysis.

- The aim is to provide information to Olivia and her husband in order to make an informed decision with regard to medication, continuing or stopping the medication.
- Explanation of risks of continuing medication during pregnancy versus the risk of relapse on stopping medication.
- Explanation that the maximum risk of malformations with exposure to medication is during the first trimester (first 12 weeks)
- By the time pregnancy is identified, this is usually at eight weeks and the maximum risk for congenital malformation has passed.
- Review current medications and cease any unnecessary medications.

3. Explanation of the risk of relapse of mental illness on foetus and mother.

- Explanation that priority is to ensure stable maternal mental health during pregnancy. Psychiatric disorders such as depression are associated with poor maternal and foetal outcome such as low birth

weight baby, neglect, poor appetite, increased risk of self-harm and suicide for the mother and, postnatally, impaired attachment and bonding that can impact on the intellectual development of the child.

4. Explanation of risk of continuing medication in pregnancy.

Antidepressants:

- Olivia is on paroxetine. Paroxetine - persistent pulmonary hypertension in the newborn with exposure in the third trimester.
- Sertraline is considered safe as it has the least passage through the placental barrier.
- Escitalopram is also considered safe.
- Avoid tricyclic antidepressants. Be cautious about the risk of withdrawal syndrome in infant associated with the use of SSRI in the third trimester.
- Consider postnatal monitoring for at least three days after delivery to detect any neonatal withdrawal syndrome. Fluoxetine has a long half-life and therefore can result in a protracted neonatal withdrawal syndrome. Consider reduction of dose in later stages.

Benzodiazepines.

- Olivia is on diazepam. Benzodiazepines are associated with **cleft palate** with exposure in the first trimester, associated with **floppy baby syndrome** with the exposure in the third trimester.
- **Neonatal withdrawal syndrome** may be present when used in the third trimester.

Sodium valproate is associated with **neural tube defects** with exposure in the first trimester and **neurodevelopmental delay with exposure in the second and third trimester**.

- Valproate is contraindicated in women of childbearing age and therefore an alternative may need to be considered even when Olivia is not pregnant. Lamotrigine is considered safe in pregnancy. Lithium associated with risk of Ebstein's anomaly is 1:1000.
- Lamotrigine is, however, evidence-based in the prophylaxis of depressive episodes and does not have antimanic properties. Lithium has anti-manic properties.

5. Breastfeeding.

- All medications passed through the breast milk to some extent. **Sertraline as an antidepressant has least passage through the breast milk.**
- Consideration should be given to formula feeds, timing the feeds, expressing breast milk if medication that passes through the breast milk needs to be continued.

6. Specialist services.

- Liaison with obstetrician, general practitioner, Perinatal Mental Health Service, explanation of **specialised perinatal mental health services** in the context of increased risk.

Migrant / Asylum Seeker Cultural Assessment and Management Principles

Question 4: Emile, a refugee from Serbia came to Australia 20 years ago. His parents and younger brother were killed eight years ago while seeking asylum before coming to Australia. He witnessed multiple atrocities and trauma. He lives with his wife and two teenage children at home. His general practitioner referred him to you as he has been feeling teary with chronic headaches and sleep disturbances. His general practitioner has investigated his headaches. Outline your differentials.

Answer:

[The important aspect here is to be relevant to the question. One does not have to think in typical DSM-5 terms but to think in terms of what may be the relevant diagnostic formulation for Emile. As each domain is written, the specific buzzwords that help you rule in or rule out the diagnosis should be written next to it.]

1. Depressive disorder.

- Emile may have experienced significant losses after his arrival to Australia in forced circumstances. His chronic headaches and sleep disturbance may be manifestations of somatic symptoms and neurovegetative symptoms of depression. He is also feeling teary.
- Culturally appropriate expression of psychological distress as somatic symptoms

2. Chronic adjustment disorder.

- Emile may have experienced significant adjustment issues such as loss of role, family and **untreated grief** that may result in a chronic adjustment disorder currently.

3. Chronic pain disorder, ie, headaches due to insomnia.

4. Posttraumatic stress disorder.

- Insomnia may be due to nightmares. He has witnessed multiple atrocities and trauma acting as a risk factor for the development of posttraumatic stress disorder.

5. Organic conditions. (e.g Obstructive sleep apnoea, head injury untreated)

- Organic evaluation to avoid diagnostic overshadowing, (attributing somatic symptoms to psychological/cultural factors only).
- Relevant aspects include ruling out obstructive sleep apnoea resulting in sleep difficulties and headaches and an MRI for consequences of head injury.

6. Substance-use disorder. Substances may be used as part of coping with current stressors, alcohol, pain medication.

Outline your assessment process.

Answer:

[Here the question that should be asked is, what is different about this case? We all know that one can write about a typical assessment formula; however, here one could ask, what is different that needs to be considered in interviewing a migrant or an asylum seeker or a refugee?]

- 1. Awareness that the patient may be guarded and information may need to be obtained after establishing an adequate therapeutic alliance. Collateral information may be required.**
 - Collateral information may be important as part of the assessment. There is a risk of disengagement, risk of mistrust and there may be associations of hospital or authority figures with trauma.
- 2. Awareness of cultural issues and cultural sensitivity.**
 - **Involvement of a mental health multicultural team with specialised expertise** [in the assessment of refugees, migrants or asylum seekers.]
- 3. Involvement of a translator if language difficulties or patient wishes. (awareness of confidentiality issues as the patient may be living in a small community)**
- 4. Risk assessment -**
 - The patient may be guarded and therefore a high index of suspicion would be required in assessment with regard to the risk of harm to self or others.
- 5. Cultural assessment:**
 - A cultural assessment would include specific aspects such as language, ethnicity, pre-migration role, employment, social status, reasons for migration, trauma during migration and post-migration losses: of family role, finances, etc. Acculturation and acculturative process
- 6. Evaluation of psychiatric disorders.**

- **Psychiatric disorders may present atypically.** e.g: Depression may present with predominant somatic symptoms. A high index of suspicion for substance use as a coping mechanism.

7. Psychosocial evaluation.

- Finances, relationships, vocation and accommodation stability.
- Current **visa status of self and other family members** that may act as additional stressors.

8. Medical evaluation to avoid diagnostic overshadowing:

- MRI, investigations of obstructive sleep apnoea, thyroid dysfunction that may be associated with depression **and posttraumatic stress disorder, nightmares, avoidance phenomenon, hyperarousal symptoms, aggression.**

Question 6: What collateral information would you require? (4 marks)

Answer:

1. Obtain info from wife, children, GP or previous notes

2. **Evidence for risk.** Risk of harm to self or others, risk of violence, aggression, risk of harm to family members.

3. Evidence for psychiatric disorders.

- Hyperarousal at night, shouting in bed, thrashing in bed,
- avoidance phenomenon of traumatic triggers, for example, not watching TV, listening to music, engagement with cultural groups
- depression, tearfulness, social withdrawal, family withdrawal.

4. **Evidence for substance use.** Use of substances culturally appropriate, traditional medications, traditional doctors, belief systems,

5. **Evidence for psychosocial adjustment issues,** finances, relationships, vocation and accommodation.

Question 7: During your assessment, you found no psychiatric diagnosis. He states that his wife has been busy with her work and his two teenage sons that are not interested in cultural activity which he finds very frustrating. Outline your management plan.

Answer: [Again here, it is best to consider the family as a unit and divide the interventions for different family members.]

1. Referral to a psychologist for Emil for **supportive counselling**, taking into account cultural considerations. **Help with cultural shock**
 - Although Emile does not have a psychiatric diagnosis, supportive counselling, problem-solving advice may help.
2. **Providing information about local groups.** e.g. Cultural resources, men's shed or local migrant groups.
3. **Meeting with the family.** Psychoeducation to the family about Emile's difficulties, enhancing communication and bonding.
4. **Involvement of an occupational therapist for vocational rehabilitation.** This would empower Emile to take up activities or gainful employment, decreasing the feelings of isolation.
5. Psychoeducation to Emile about cultural differences and the importance of the children developing independence in a different culture and to facilitate the acculturation and acculturative process.
6. Other psychosocial interventions. Example: **Financial.**
7. If relationship conflict present – relationship counselling

Culture Shock:

A psychological disorientation experienced when individuals encounter a culture markedly different from their own. It often occurs in migrants, refugees, international students, or expatriates.

Stages (Oberg, 1960):

Honeymoon phase – fascination, excitement
Negotiation phase – anxiety, frustration (language, customs)
Adjustment phase – problem-solving, gradual adaptation
Mastery phase – bicultural competence, integration

Acculturation:

A process by which individuals adapt to a new culture after migration, which may involve:

Assimilation (adopting host culture fully)
Integration (keeping original culture while adapting to new one)
Separation (maintaining heritage, avoiding host culture)
Marginalization (loss of both cultures)
Berry's Acculturation Model is often cited in psychiatric literature.

Therapeutically Encouraged	Should Be Avoided
Healthy acculturation	Forcing **assimilation**
Integration** of both cultures	Cultural invalidation**
Coping with new cultural stressors	Pathologizing cultural expression
Building bicultural competence	Ignoring **cultural bereavement**

Behavioural Disturbance in Elderly - Dementia

You have recently assessed Mr Newbury, aged 84 years who lives in a residential care centre for people with dementia. Mr Newbury suffers from **moderately severe Alzheimer's disease**. The staff are concerned that he is **verbally aggressive towards staff and other residents** and occasionally **lashes out and hits staff**. Some of the untrained staff in the centre feel that Mr Newbury is deliberately playing up and are annoyed with him. When you examine Mr Newbury, he is calm but severely cognitively impaired. He has no apparent physical problems. Recent routine lab investigations including midstream urine culture unremarkable. Mr Newbury is on no medication.

Question: Describe the information you would ask residential care staff to collect regarding Mr Newbury's difficult behaviour?

Answer:

1. **General behaviour and baseline functioning:**
 - Sleep, appetite, speech verbal or through gestures, daily routines, daily activity, engagements in daily routines and activity, number of staff, number of residents, male-to-female ratio, staff-to-patient ratio.
2. **ABC analysis (antecedents, behaviours and consequences).**
 - **Antecedents:** Triggers to aggressive behaviour, time of the day, sporadic or a pattern identified, e.g. towards a specific staff member or specific resident, response to diversion or commands.
 - **Behaviours:** Specific description of the behaviour, how long the episode lasts, other behavioural and psychological symptoms of dementia such as disinhibition, confusion, self-harm, wandering, absconding.
 - **Consequences:** How do staff respond, is there punishment or positive or negative reinforcement used, consistency of the response, medications administered and effectiveness of interventions, environmental modification and psychological modification strategies used after behaviour.

3. Evaluation for Causes for behaviour

- **Medical causes:** Example, fever, pain, constipation, urinary tract infection, delirium, electrolyte imbalance.
- **Environmental changes:** Change in residence, change in staff, abusive behaviour from staff, abusive behaviour from other residents, changes in routines, changes in activity.
- **Psychological factors:** New-onset illness such as comorbid depression, comorbid psychosis or comorbid disinhibition (mania).

Question: When you return to the centre one week later, you find that staff have not collected any information for you. Discuss the factors that make it difficult for care staff to collect such information.

Answer: [When asked to review staff-related or systemic factors; a good approach is to divide this into staff factors, patient factors and system factors.

1. Staff factors:

- Lack of training about the management of behavioural disturbance of dementia
- lack of knowledge
- poor staffing resulting in an increased burden on staff making it difficult to collect information
- Negative attitude towards dementia
- nihilistic attitude that people with dementia do not get better
- judgemental attitude towards patients
- staff feeling scapegoated or blamed
- information not explained clearly
- language issues.

2. Patient factors:

- The severity of the illness/dementia
- severe behavioural and psychological symptoms of dementia
- significant aggression
- language difficulties.

3. System factors:

- Poor handover systems
- lack of electronic records
- inadequate staff
- workload is heavy
- poor documentation
- shift work
- continuity of care is poor.

Question: The management of the centre asked for a teaching session for the regular staff on management of difficult behaviour and dementia patients. Discuss how you would educate the staff.

Answer: [Here there are two aspects. They are the role of an educator and the role as a clinician.]

1. Devising and delivering a talk:
 - audience analysis
 - analysis of baseline knowledge of staff
 - a pre-educational survey asking what staff want to know
 - talk to be delivered in an interactive manner with opportunity to ask questions,
 - use case studies, practical information and provide other reading resources.

2. Content:

- Nature of dementia
- explaining that it is not the patient's fault
- Common dementias: Alzheimer's dementia, vascular dementia, Lewy body dementia.
- [Specific symptoms related to each], e.g
- Alzheimer's dementia, memory complaints, difficulty dressing, forgetfulness;
- vascular dementia, related to stroke, stepwise deterioration;
- Lewy body dementia: visual hallucinations.

3. Causes of behavioural disturbances

- medical causes
- environmental causes
- psychiatric causes.

4. Explanation about how to carry out a behavioural analysis, antecedent behaviours and consequences and ensuring there is a chart to document behavioural analysis.

5. Treatment of behavioural disturbance in dementia.

- **Environmental strategies:** Adequate routine, regular exercise, music therapy, decluttering of the ward, adequate spaces.
- **Psychological strategies:** Talking to patient in a calm manner, regular staff, consistent person, family involvement if necessary, the importance of positive reinforcement avoiding punishment, consistency of positive reinforcement.
- **Medication strategies:** Medication to be used as a last resort, risk of using antipsychotics and risk of vascular events, to be used in severe agitation at small doses.

Schizophrenia / Risk Assessment / High Expressed Emotion

Omar is a 23-year-old gentleman living at home with both his parents who are medical practitioners. He has been diagnosed with bipolar affective disorder and has a history of cannabis and methamphetamine dependence. He has been treated under CTOs twice previously where he lives in another city and is on risperidone and lithium. He is now moved back with his parents under the condition that he takes his medication. Parents are prescribing and checking his levels.

Question: What information do you need before assessing Omar?

Answer: [The pattern that the college expects tends to be fairly similar.]

1. **Risk:** [As you can see, the patient is on risperidone and lithium so risk of medication is significant.]

- Current lithium dose and level to rule out possibility of lithium toxicity,
- sensorium if collateral can be obtained and the dose of risperidone administered.
- Other risks include risk of harm to self and others in past and present and forensic history.

2. Identifying the treating team and clinician

- Is he currently under the community mental health team as a voluntary patient or under a CTO (community treatment order)
- Which jurisdiction has the CTO been implemented under. Which service has clinical and legal responsibility?

3. Past history of illness

- the severity of the illness
- symptoms when ill
- psychotic symptoms
- affective symptoms
- risk of harm to self or others.
- **Past history of compliance with medication and insight.**

5. Current phase of illness

- acute, partial remission or complete remission.
- Current symptoms if identified - psychotic symptoms, affective symptoms.

6. Substance use history -

- Current and past
- duration of substance use
- choice of substances
- temporality between substance use and the onset of illness
- intravenous use or oral
- motivation to change.

7. Psychosocial history

- Employment history
- relationship with family
- family dynamics
- expressed emotion.

8. Medical history

- Any contributing factors such as epilepsy, head injury, thyroid issues, metabolic dysfunction.

Question: If his father calls the clinic and states that he does not wish Omar to be seen by the registrar even though Omar finds the registrar useful. He wants the son to be reviewed by the consultant. How would you outline your approach to the request?

Answer: [There are several questions in the MEQ where the family member gets involved or there is some conflict between the family and the patient or the family wishes certain treatment to proceed in a certain way.]

1. **[Think patient first]** Speak to Omar, explain the father's request and ask for consent to speak with father. Respecting confidentiality.
 - This is linked to ethical principle of autonomy.

2. Ethical complications:

- beneficence: acting in the best interest of Omar
- nonmaleficence: doing no harm

[Educator and collaborator role]

3. Arrange meeting with the father:

- listen to father
- understand the reasons for the request
- acknowledge the concerns.

4. Psychoeducation for father

- Inform the father that informed consent from Omar is important and breaching confidentiality may disrupt the therapeutic alliance.
- Provide psychoeducation about the community mental health team that the psychiatrist and registrar work together, that registrar is under the supervision of the consultant.
- Inform that patient choice is an important part of management and consultant review is part of the management plan as part of the psychiatric review and therapeutic alliance is associated with good outcomes in psychiatric disorders.

5. If the father requests a meeting with the consultant/director, then discuss with the consultant or arrange a meeting with the consultant for further clarification.

Question: Father wants Omar to have more intervention and feels that not enough is being done for him. He has read about IPSRT and wants to find out how this would be helpful for Omar. Describe what it is and how it might benefit the patient?

Answer:

[When psychological therapies have been asked in relation to a specific patient, it is important to not only describe the psychological therapy in generic format but then also to address it in relation to the patient.]

Description of Interpersonal social rhythm therapy (IPSRT):

- IPSRT consists of interpersonal psychotherapy combined with social rhythm aspects.
- This is based on the principle that the interpersonal changes in a person's life along with circadian rhythm disturbances can act as a risk factor for the relapse of bipolar affective disorder.
- Interpersonal psychotherapy consists of addressing any of the following - role transitions, role disputes, interpersonal deficits or grief.
- Social rhythm therapy is related to the importance of maintaining a routine in individuals with bipolar affective disorder as disruptions of

routine, e.g. sleep-wake cycle, mealtimes, etc, can disrupt circadian rhythms that can predispose an individual to relapse.

Applicability to Omar:

- In Omar's case, the grief of having an illness, role transition, moving to parents' house indicate that he would benefit from interpersonal psychotherapy and the circadian rhythm disturbances, the importance that routine poor sleep would need to be addressed as part of relapse prevention in bipolar affective disorder.

Question: Father has now prescribed and administered a depo, how would you handle the situation?

Answer: [In this case, this can be addressed as a CanMEDS approach. When we come across a difficult situation, think CanMEDS].

Autonomy, beneficence, non-maleficence

1. Professional issues/ medico-legal issues: [Think patient first]

Examine the patient, rule out severe side effects such as parkinsonian side effects or neuroleptic malignant syndrome, check the dose of the medication,

Boundary violation:

- Prescribing medication to family members is considered as acting outside the scope of practice.

Beneficence:

- Prescription of a depo is a specialised treatment and requires the involvement of a specialist team.

Nonmaleficence:

- Prescribing of depot unsupervised can result in significant harm.

2. Arrange a meeting with both father and mother [Collaborator & Educator]

- Explanation about boundary violation
- clear advice to the father to avoid any further prescribing
- advice to father to contact medico-legal insurance
- informing the father that medico-legal obligations of mandatory reporting may need to be considered.

3. Seek advice by contacting medico-legal insurance about mandatory reporting

Organic Presentation

Question: You are a consultation-liaison psychiatrist. You are referred Bill by the Endocrinology Team. Bill is a 45-year-old married electrician who has a history of pituitary carcinoma. His tumour was successfully removed by the Endocrinology Team; however, since that time, Bill has developed mood lability, social withdrawal and low self-esteem. He initially returns to work as an electrician; however, left his work after six months as he lost confidence. He also suffers from chronic headaches and insomnia. Bill has sleep apnoea and uses CPAP machine. He is managed with thyroxine and corticosteroids.

Outline the reasons for Bill's mood lability? (6 marks)

Answer: It is important to look at the specific aspects related to Bill.

1. Endocrine related issues:

- Removal of pituitary tumour may have resulted in a reduction in secretion of pituitary hormone; therefore, possibility of Addison's disease resulting in mood symptoms and inadequate supplementation with thyroxine and corticosteroids.
- Over supplementation leading to Cushing's syndrome which can be associated with mood disorder.

2. Untreated sleep apnoea resulting in insomnia and chronic headaches.

3. Chronic headaches may be related to possibility of undetected metastases.

4. Depression secondary to corticosteroid medication, corticosteroid-induced mood lability and insomnia.

5. Cognitive impairment resulting in difficulty working and secondary depression due to thyroid dysfunction but also post-surgery inflammation.

6. Substance use as a coping mechanism.

7. **Adjustment disorder** related to surgery and loss of work presenting with mood symptoms.

8. **Recurrence of tumour** resulting in headaches and mood symptoms.

9. **Exacerbation** of an underlying mood disorder.

Question: You meet with Bill and his wife. Bill's wife has taken over cleaning, shopping and management of bills since Bill's surgery. Bill feels angry and frustrated about this. According to the occupational therapist, Bill is capable of doing these things himself.

Outline the management of the situation.

Answer: [One can think in terms of the CanMEDS model. You can see three issues that are evident here]:

[The autonomy of the patient and the patient's wishes to engage in activity but also to consider the educator and collaborator role with both Bill and his wife].

1. Arrange a meeting with Bill and his wife:

- Understand the reasons why Bill's wife has taken over cleaning, shopping and management of the bills as this may be related to anxiety from his wife wanting to help Bill
- Understand shared goals, hobbies
- psychoeducation to his wife that empowerment and independence will assist Bill in improving his self-esteem and may help in treating the depression.

2. If significant carer burden is identified, then involve a social worker to provide social support, carer support for wife.

3. Consider a neuropsychological assessment in relation to Bill's capability about doing tasks to obtain an objective measure.

4. Bill's decision-making capability with regard to medications, involvement in activities.

5. If any relationship conflict identified, then consider relationship counselling and consider carer groups or patient groups for wife.

Question: The Endocrinology Team tell you that Bill's tumour has returned although it is treatable. Bill has become increasingly irritable and labile in his mood.

Outline the possible causes for Bill's mental state deterioration. (4 marks)

Answer:

[Although we would think about the pituitary carcinoma related issues, one also has to think about the adjustment issues that might be a play here.]

1. Pituitary carcinoma, endocrine disturbances: Cushing's, hyperthyroidism, hypothyroidism.
2. Frustration and depression related to adjustment and grief.
3. Fear of death, fear of cognitive decline, fear of surgery
4. **Financial stressors:** costs associated with surgery resulting in financial difficulty, fear of lack of support for the wife and carer burden.
5. Adjustment disorder or grief reaction in response to the diagnosis

Doctor and mental illness / Impaired doctor

MEQ7: A 35-year-old, Janine who works as a Career medical officer at the Emergency Department was referred to you by general practitioner for admission with five weeks' complaints of low mood, sleep problems and issues with concentration. The general practitioner stated two past episodes of depression which were treated with antidepressants (eight to nine months each time). He also noted that she recently had a complaint from the patient in the Emergency Department and had to take a week off.

The director of ED called you to discuss her admission. He tells you of the complaint made by a patient against the chief medical officer (CMO) of acting inappropriately.

List and explain ethical and work issues.

Answer:

1. **Autonomy:** Informing the patient depending on level of unwellness and preferably when stable, of the complaint that has been made. This is related to natural justice.
2. **Duty of care towards patient:** If patient harm has been identified, then awareness of mandatory reporting obligations to the medical board, discuss with medico-legal insurance. Follow local notification guidelines related to the service.
3. **Patient's confidentiality:** One cannot discuss the patient's details with the director of ED without the patient's content. This will be considered as a breach of confidentiality.
4. **Beneficence and non-maleficence:** Acting in the best interest of the patient, taking into consideration risk of harm to reputation, be sensitive about the location of admission, preferably in a private hospital at a site distant from where the doctor works.
5. **Return to work** with gradual support, involvement of the human resources and occupational health and safety via a support group, addressing any stressors that would act as barriers with return to work.

Delirium Psychogeriatrics

MEQ8: You are called to carry out a home visit at Mrs Smith's house, who is 72 years of age. She presents mildly confused, irritable and unsteady on her feet. Her previous mini-mental state examination scored 21/30. She showed impaired attention, concentration and short-term memory. She is currently on atenolol, lithium, fluoxetine and L-DOPA. Outline the possible causes for her presentation.

Answer:

1. Delirium:

- urinary tract infections, chest infections, common causes of delirium in the elderly as justification.
- 2. **Hyponatraemia** due to syndrome of inappropriate ADH secretion due to prescription of fluoxetine and lithium.
- 3. Atenolol being a beta-blocker may be prescribed for atrial fibrillation. Delirium may be due to **pulmonary embolism or cardiovascular stroke events** due to uncontrolled atrial fibrillation or other arrhythmias.
- 4. Parkinsonism-related aspects: L-DOPA-related mood changes, excess dopamine resulting in confusion, irritability, unsteadiness on feet maybe due to inadequately treated parkinsonism.
- 5. Lithium toxicity, due to forgetfulness and accidental overdose.
- 6. Abrupt cessation of lithium resulting in rebound confusion.
- 7. Serotonin syndrome due to a combination of fluoxetine and lithium can result in confusion and unsteadiness.
- 8. Abrupt cessation of SSRIs can result in withdrawal, dizziness, unsteadiness in feet.
- 9. Possibility of Lewy body dementia which can present with fluctuating cognition, persistent vivid visual hallucinations in the context of Parkinsonism.

10. Other substance use, such as alcohol withdrawal or alcohol intoxication.

Question: Mrs Smith is admitted to the Older Patients Mental Health Service Inpatient Unit. What would be your advice to the registrar in prescribing of medications?

Answer:

1. Start low and go slow.
2. Optimise the medications and cease unnecessary medications.
3. Liaise with the neurologist with regard to look at optimising Parkinsonian medication, liaising with the cardiologist to optimise cardiology medications.
4. Carry out lithium levels to rule out lithium toxicity.
5. Assess for compliance.
6. Rule out medical causes for confusion and delirium, urinary tract infection, CRP levels for inflammation, hyponatraemia, urine osmolality and serum osmolality to rule out SIADH.
7. Rule out serotonin syndrome due to myoclonus confusion, hyperreflexia.
8. Liver function tests and renal function tests as medication dose may need to be adjusted if deranged.
9. Extreme caution with regard to use of antipsychotics in the context of parkinsonism that can result in worsening of parkinsonian symptoms if Lewy body dementia is present, increased risk of mortality due to hypersensitivity with antipsychotics. Antipsychotics can also have anticholinergic effects worsening confusion.
10. Benzodiazepines can increase the risk of falls and confusion.
11. Caution about other drug-drug interaction such as prescription of ACE inhibitors and lithium, NSAIDs and lithium.

Question: Her son is visiting from overseas and he wants her to make a Will.

Outline your process of assessing testamentary capacity.

Answer

1. **Patient autonomy and capacity:**
 - Is the patient currently well and stable?
 - Inform patient of the request from son.
 - Obtain the patient's wishes.
 - If the patient is unwell, the assessment of testamentary capacity if deemed essential would need to be deferred.

2. Components of testamentary capacity:

- Understanding and retaining information.
- Decisional balance weighing up the pros and cons of making a decision.
- Communicating the decision.
- Freedom from undue influence.
- Consistency.

3. Specifics of testamentary capacity: The nature and effect of making a Will

- The extent of her estate
- who might be the reasonable beneficiaries of her assets
- how she wishes to distribute her assets
- the understanding of the impact of the distribution of assets
- freedom from undue influence, freedom from delusions impacting on her decision making, freedom from cognitions that impact on her decision making, freedom from delirium that impact on her decision making.
- consistency to re-evaluate after a period to ensure that the decision is consistent.

4. Record verbatim answers, check facts with solicitor, ask about previous Wills, why potential beneficiaries have been included or excluded during assessment.

Assessment of enduring Power of Attorney.

Answer:

There are two types of powers of attorney, General Power of Attorney and Enduring Power of Attorney. A General Power of Attorney is made so that someone can make financial decisions on behalf of a person when they were absent or if they simply prefer another person to make these decisions for them.

An Enduring Power of Attorney appoints someone to begin or continue to make decisions for a person when they no longer have the capacity to make their own decisions about the financial affairs.

Components of capacity (see previous)

In making a Power of Attorney, understanding and retaining the following information:

1. Do they understand that they are appointing someone to make financial decisions on their behalf?
2. Do they understand that they can appoint more than one attorney?
3. Do they know that when they appoint the Enduring Power of Attorney, the EPA will begin making decisions or continue making decisions when the person lacks the capacity to make financial decisions?
4. Does the person understand the effect of the document, that attorney can make decisions about the things detailed in the Power of Attorney document, these are legally binding decisions on their behalf?
5. They can cancel the Power of Attorney or change it in the future as long as they have the capacity.
6. If the power is an Enduring Power of Attorney, the attorney will continue making decisions for them as long as they lack the capacity to do it for themselves.
7. If it is an Enduring Power of Attorney, they cannot revoke or change the appointment while they lack capacity.
8. Freedom from undue influence: Can they communicate the above with assistance if necessary?

Child and Adolescent Psychiatry - Encopresis

Question: You have been referred to see a 6-year-old boy, Sam, by the paediatrician for encopresis who lives with his mother who is a single mother. The paediatrician has ruled out any organic issues and started him on laxatives and a star chart which has not worked. Outline your assessment of encopresis.

Answer:

1. Multimodal assessment

- Assessment of the child.
- Assessment of the parents.
- Assessment of the child interaction with the parents
- obtaining information from other sources such as a paediatrician, general practitioner, school observation, school psychologist, mother and any other family members involved

2. Liaise with the paediatrician to identify the organic conditions ruled out.

- Ensure neurological conditions, Hirschsprung's disease, overflow incontinence ruled out.

3. Assessment of encopresis -

- Onset, duration, progress, and aggravating factors/relieving factors.
- Child is usually continent of faeces by 4 years of age.
- Antecedent, Behaviours and consequences: What happens before the encopresis, is it at the specific time of the day, specific location, is it pervasive across the setting, how many times during the day, any associated behaviours with the encopresis, aggression, agitation, anxiety. Consequences - how does mother react, how does school react if it happens at school, any punishment, negative reinforcement or positive reinforcement?

4. Assessment of comorbidities:

- Pervasive developmental delay, social interactional deficits, stereotypical and ritualistic behaviour.

- Learning difficulties: IQ testing, evidence of anxiety such as school refusal, separation anxiety, teariness, crying, sleep disorders, nightmares.

5. Developmental history:

- Mother's pregnancy
- complications during pregnancy
- complications during delivery
- exposure to substances or smoking during delivery
- early childhood illnesses
- infections
- neurological conditions
- delay in achieving milestones indicative of pervasive developmental delay.
-

6. Psychosocial history:

- Relationship with mother and father
- any domestic violence
- any exposure to violence
- risk of harm to the child
- mother's maternal health
- financial, relationship, vocational and accommodation instability contributing to stressors to the mother.

7. Reasons why star chart has not worked

- Maternal factors: lack of belief in star chart, inability to implement star chart consistently, inadequate training regarding the use of star chart administration of laxatives too frequently, noncompliance with use of laxatives or star chart. Mother not having enough time or support.
- Child factors: Oppositional defiant behaviour, aggressive behaviour.

Sam has not seen his father for some time, mother has depression and is on antidepressants. His mother said that she is tired of bribing him to behave. When he sees Sam, you notice that he has a bruise on his arm. Outline your management plan.

Answer:

1. **Rule out severe injuries** by involving an emergency department physician or a paediatrician for a full physical examination. The bruise may be indicative of a more severe injury.

2. Involve a health professional with expertise in the assessment of child abuse, Child Protection Services, social worker, Mental Health Service and child and adolescent psychiatrist.
3. **Evaluation for suspicion of abuse.**

Communication with the child:

- Stay calm.
- Communicate in a way that is age and developmentally appropriate.
- Provide the child with a private and child-friendly environment.
- Respond in a sensitive and caring manner.
- Provide support without being judgemental.
- Listen to what the child has to say first.
- Use open-ended questions.
- Avoid leading questions.
- Do not probe for details.
- Do not promise confidentiality.
- Minimise the number of individuals examining the child.

Communication with the parent:

- Adopt a nonjudgmental manner.
- No leading questions.
- Empathise but do not condone abusive behaviour.
- Clear advice of doubt of what constitutes abusive behaviour and keep the parents informed if the child needs at all time clear documentation.

4. If risk of harm to the child has been identified, then consider the involvement of mandatory reporting obligations to Child Protective Services and their involvement, involve other legal guardians, social workers.

5. If no significant risk of harm to child has been identified, child can be discharged with the mother, then consider post-discharge followup from Child and Adolescence Services.

C-L Psychiatry - Complexity

A 4-year-old girl who lives with her father and her mother and has two older siblings, aged 8 and 10, was admitted to the medical ward for investigation of her abdominal mass which turned **out to be malignant on histology**. Her father became irritable after being told the news, stopped sleeping and started responding to hallucinations and behaving bizarrely as per nursing staff. Currently, he is by daughter's bedside, agitated, verbally aggressive to the staff, not letting blood samples be collected. Nursing staff call you from the ward.

What information would you get from the treating team?

Answer: [When you come across a conflict situation, again CanMEDS can be helpful]

1. Who is the legal guardian? Is another parent involved? What are the legal guardian's wishes?
2. Is the child in danger from physical risk?
3. Is the blood test urgent or can it be delayed?
4. What de-escalation techniques have been considered? e.g meeting with father, explanation of daughter's illness and prognosis
5. Is the patient intending to harm self or others? Has security have been informed? Do police need to be involved?

List and explain the possible causes for the behaviour of the father.

Answer:

The parent (patient) factors:

- Acute stress reaction.
- Exacerbation of underlying psychotic illness due to stressors.
- Frustration and anger with treating team.
- Substance use-related irritability and anger due to substances being used as coping mechanism.

Staff factors:

- Poorly broken bad news.
- Lack of time given to father to ask questions
- Father has been unable to vent anger and hostility.

- Lack of cultural sensitivity.

System factors:

- Transference and countertransference issues related to the team.
- Lack of continuity of care.
- Regularly changing staff, resulting in inability to build a therapeutic alliance.

In this case, list and outline the ethical and medical issues.

Answer:

1. Beneficence: Acting in the best interest of the child- is carrying out a blood test in the best interest of the child; reasons for blood test.

2. Nonmaleficence: Could delaying the blood test result in harm to the child?

3. Autonomy and capacity: The child is unable to make a decision for self; therefore, proxy guardian required who is the legal guardian

- If father is a legal guardian and is not acting in the best interest of the child, consider obtaining consent from other guardian.
- If both legal guardians are not acting in best interest of the child, seek advice from the emergency guardianship.
- Seek advice from medico-legal insurance.

4. Risk and mandatory reporting: Since child is involved, duty of care to child and mandatory reporting obligation awareness if risk of harm to child has been identified.

5. Duty of care to father: As father is ill, involvement of Mental Health Team to carry out assessment and treatment as required.

Chronic Fatigue Syndrome / Sleep / Adolescent Psychiatry

You are a junior psychiatrist and Virginia is a 14-year-old girl who has been referred to your community mental health service by her family doctor who has diagnosed her as having chronic fatigue syndrome. Virginia has a two-year history of excessive tiredness and nonspecific physical symptoms. Virginia has had homeschooling for the past year, avoids social settings and has a little contact with peers. She says she could do all these things "if they were open at night when I am awake". She stays up watching television and talking to online internal chat rooms until 3 am and then sleeps until midday. Virginia's mother, Mrs Bennett, has stopped work so she can care for her daughter. The family doctor says he has excluded all significant organic causes of Virginia's fatigue but wonders if there are psychiatric problems. He requests that you assess and treat her. You take her history and carry out her mental state examination.

Outline the areas of history that you would focus on in assessing her presentation. (15 marks)

Answer:

1. Organic issues:

- Blood test for anaemia
- vitamin B12/folate deficiencies
- vitamin D deficiency
- thyroid dysfunction: especially hypothyroidism
- dietary intake.

2. Assessment of sleep hygiene and routine:

- Time she goes to bed
- presence of early, middle or late insomnia
- use of devices in bed
- stimulant intake during the day e.g. Red Bull and Monster stimulant drinks,
- regular routine or erratic sleep pattern.

3. Evaluation of fatigue:

- Nature of its onset, duration and progress.
- Any specific pattern, all day or end of day.
- Any specific body aches and allodynia indicating the presence of fibromyalgia, fatigues worsening on exertion indicating chronic fatigue syndrome
- aggravating and relieving factors.

4. Evaluation of comorbidities:

- Mood symptoms: presence of melancholic features, neurovegetative symptoms, example, loss of weight, loss of appetite,
- presence of mixed features such as racing thoughts can be associated with sleep difficulties.

5. Evaluation of anxiety:

- Generalised anxiety symptoms related to school or other stressors such as bullying
- presence of specific phobias
- presence of panic attacks
- school anxiety disorders
- presence of school refusal.

6. Developmental history:

- Presence of attachment difficulty
- separation anxiety as a child
- child anxious temperament
- anxious/ avoidant trait
- evidence for abnormal illness behaviour in parents
- conditioning phenomenon resulting in nurture or care through illness as a learned behaviour
- history of parental physical illness
- high expressed emotion [over-involvement, parental criticality or hostility and dependency.]
- Presence of developmental delay or attention/concentration deficits indicative of ADHD resulting in school difficulties.

7. Psychosocial development:

- Peer group relationship
- Academic performance.

Outline differential diagnosis you would consider?

1. **Poor sleep hygiene:** Erratic sleep pattern, use of devices in bed, lack of routine during the day.
2. **Abnormal illness behaviour:** Secondary gain for the child, obtaining nurture and care through physical illness.
3. **Secondary gain** for mother through child's illness, codependent relationship.
4. **Organic illness** not evaluated such as thyroid dysfunction, hormonal-related mood changes, anaemia and fibromyalgia.
5. **Underlying circadian rhythm disturbance**, the parasomnias or circadian sleep rhythm disturbance disorder.
6. **Underlying mood disorder** such as depression or mixed features prodromal type.
7. **Psychosocial stressors**, bullying at school, learning difficulties, poor relationship with teachers or with parents, relationship conflicts between parents and chronic fatigue subsequent to a viral infection.

Mother asked for a sleeping tablet. Outline your advice.

The general advice about sleep hygiene, sleep hygiene recommendations, maintaining a sleep diary, stimulus control and sleep restriction.

Psychoeducation :

- about circadian rhythm disturbances and the importance of a sleep routine.
- Sleeping tablet may be utilised for short-term purposes if other measures do not work as a last resort.
- Psychoeducation that sleep medication such as benzodiazepine may have dependence potential.

CBT (cognitive behavioural therapy) for insomnia with a referral to psychologist.

Sleep study, if the sleep hygiene does not assist.

Melatonin (Circadin 2 mg) can be utilised on a short-term basis one to three months.

Finally, if does not improve, sleep specialist input,

Input of diet and lifestyle changes.

Severe Depression and ECT

A 67-year-old lady with recurrent treatment-resistant depressive disorder, presents with severe depressive episode of two months' duration. She has developed nihilistic delusions and is very agitated over the last two weeks. She has been on escitalopram 20 mg per day for the last two years. Husband and daughter are aware. She is fearful of and has previously been adamant in her refusal of ECT. Her brother died of suicide, aged 20, after having ECT for depression. Her husband and daughter feel ECT is not appropriate and objects strongly to this treatment. On the inpatient unit, she is initially commenced on venlafaxine, which was titrated to 225 mg mane over a period of three weeks with no effect. Subsequently, she was crossed titrated with nortriptyline with a dose of 100 mg being maintained over two-week period as a mood stabiliser.

Outline the professional and ethical issues raised by this scenario.

Answer:

1. Autonomy:

- The patient may or may not be able to consent to ECT due to lack of capacity. This may be related to the severity of illness; therefore, assessment of decision-making capacity, explore the patient wishes.

2. Privacy and confidentiality:

- Respecting confidentiality of the patient and obtaining consent to speak to family members.

3. Beneficence:

- Acting in the best interest of the patient/ providing duty of care.
- The nature of the depression is melancholic and the patient has psychotic symptoms. Psychotic and severe melancholic depression is associated with a high risk of suicide. ECT is an evidence-based treatment for treatment-resistant depression and has a high response rate in psychotic depression up to 95%. Therefore, the medication may be in the best interest of the patient.

4. Nonmaleficence:

- Withholding ECT may increase the risk of suicide and may cause harm to the patient.
- ECT can be associated with cognitive side effects that may be permanent in approximately 12% of cases with bitemporal administration.

5. Look at advanced directives and previous wishes if family refuses.

6. Second opinion, Mental Health Act legislation to be considered if fulfils criteria for significant risk and the patient lacks capacity.

Outline the factors that could be contributing to her depression failing to improve.

Answer:

1. The nature of the depression is of treatment-resistant depressive nature associated with nihilistic delusions and agitation, which indicates a psychotic depression
2. Psychotic depression requires broad-spectrum antidepressants such as SNRIs or tricyclic antidepressants at higher doses (dopaminergic and noradrenergic antidepressants). She is prescribed venlafaxine 225 mg mane, which is for three weeks, which indicates a subtherapeutic dose and a subtherapeutic duration.
3. Nortriptyline is a tricyclic antidepressant prescribed at 100 mg, which is of subtherapeutic dose [dose increased to 150mg nocte].
4. The Mood stabiliser may not be an antipsychotic medication and antipsychotic medication is required for treatment of psychotic depression.
5. Other factors may include severe vasculopathy as treatment-resistant depression and psychotic depression may be associated with vascular features.
6. Organic factors such as hyponatraemia, thyroid dysfunction, Addison's disease or other organic issues such as a brain tumour considering the age would need to be ruled out or a lung malignancy would need to be ruled out.
7. Other differential diagnoses such as neurocognitive impairment or prodromal dementia may contribute to treatment resistance and then non-compliance with medication.

1. Psychological factors may act as barriers, however less likely considering the nature of the depression which has a higher biological weighting.

Question: Outline the factors that maybe contributing to family's opposition to ECT.

Answer: [Here again, one can think in terms of the patient factors, family factors and treatment factors.]

1. Stigma against ECT due to media perception.
2. Negative attitude to ECT due to negative past experiences, the suicide of brother after ECT.
3. Lack of knowledge about modern techniques of ECT.
4. Psychoeducation not provided about the benefits and risks about ECT in a detailed manner.
5. Concerns about side effects of the ECT, permanent memory deficits, explanation not provided about different electrode placements and techniques to minimise cognitive deficits.
6. Family may be respecting the patient's wishes.
7. Poor engagement with treating team or treating psychiatrist, transference and countertransference issues.
8. Cultural factors or beliefs in alternate models of care

After a lengthy discussion with the patient and her family, the plan is made to begin a course of right unilateral ECT while optimising the dose of nortriptyline to 150 mg daily. By the sixth treatment, however, some signs of hypomania are evident. Describe the management you will recommend. Outline the possible treatment options at this point. Note that these are options. There is no one step that can be taken. Your aim is to outline the various options that you can put in place.

Answer:

1. Stop any further ECT, wait and watch.
2. Space out the treatments of ECT and monitor closely for worsening of hypomania or morphing of hypomania into mania.
3. Reduce the dose of nortriptyline to 50 mg with the aim of ceasing it, gradual cessation due to risk of anticholinergic rebound.
4. Optimise mood stabiliser if present for treatment of mania.
5. If antipsychotic mood stabiliser has been prescribed, increase the dose and optimise the dose of antipsychotic mood stabiliser in the context of mania.
6. Lithium augmentation is an evidence-based strategy in both treatment-resistant depression and the treatment of hypomania.

Community Treatment Order and Schizophrenia

You are a junior consultant psychiatrist working in the community adult mental health clinic, you are approached by a case manager, about Jarvis, a 28-year-old male, who's case managed on a community treatment order following a first episode psychosis when he was 22. He works in the local radio station as a jockey and lives with his parents. He is prescribed paliperidone monthly depot over the past four years. His case manager tells you he is due for a review as his CTO expires in four weeks and needs to be extended.

Question:

1.1 What information would you need from the case manager to assist you before your first appointment with Jarvis? List and justify (11 marks).

Answer:

1. Risk assessment

- Previous risk of harm to self and others.
- Current risk
- Current risk of harm to self or others.
- Dose and side effects from paliperidone needing urgent attention

2. Past history of mental illness:

- Severity of illness to justification for the Community Treatment Order and depot medication.
- Substance use history in the past.
- Have alternatives for Community Treatment Order been considered
- Have other least restrictive options, such as community treatment team, assertive outreach team, case management approach, resulted in failure and non-engagement with services.

3. Nature of the illness

- Schizophrenia or schizoaffective disorder.

- Last episode of unwellness.
- Specific symptoms - delusions, hallucinations, aggression, negative symptoms

4. Specific points related to the Community Treatment Order:

- past compliance with medication
- past engagement with services
- history of refusal to accept treatment
- relapse after non-compliance with medication followed by mental and physical deterioration resulting in hospital admission
- hospital re-initiation of medication resulting in improvement of symptoms of mental illness

5. Current mental state examination:

- Presence of current psychotic features.
- Presence of current depressive and/or manic features. Presence of anxiety.
- Risk of substance use.
- Current active suicidal ideation, intent or plans.

6. Current insight and capacity to consent for treatment:

- Does the patient have the capacity to consent for treatment?
- Does the patient believe that the treatment is beneficial?
- Does the patient consent to taking a depot medication?
- Does the patient have expressed the choice for oral medication?

7. Psychosocial supports:

- Current relationship with parents.
- Vocational stability
- Relationship stability
- Accommodation
- Finances.

(These are factors that, if present, would be associated with a positive outcome with Community Treatment Order and depot medications.)

Question: What information from the review would impact on your decision? List and justify (10 marks). (Decision for CTO)

Answer:

- Previous risk of harm to self or others when unwell.
- Refusal to accept treatment and the patient lacks capacity to make the decision regarding his treatment.
- The patient has a treatable mental disorder, such as psychosis/schizophrenia.
- The patient does not want to continue to take medication, which is likely to help improve symptoms and reduce the risk of relapse.
- Previous treatment in hospital has not been sufficiently effective to lead to remission or improvement
- Discharge from hospital has resulted in relapse.
- Patient understands the role of a Community Treatment Order and accepts the conditions of the Community Treatment Order.
- Community Treatment Order is the least restrictive option for the patient.
- The patient does not currently need to be admitted to hospital ie. does not have active symptoms of mental illness that need to be treated in hospital and does not have current risk of harm to self or others that require immediate admission.
- Patient lacks insight into his illness and has poor compliance with medication, both past and currently refuses medication.
- The illness has significantly impacted on his psychosocial functioning, employment, relationships or accommodation.
- Patient chooses a community treatment order

Question: A medical student on placement asks you what the evidence is for the use of Community Treatment Orders. Explain your response. List and elaborate (6 marks).

Answer:

A Community Treatment Order is a legal framework under which a patient can be stipulated to take medications, attend appointments to enhance compliance of medication to treat the mental illness and to prevent the risk of harm to self and others and to assist with recovery.

Evidence for Community Treatment Order:

- It may be justified on ethical grounds as a Community Treatment Order may be in the best interests of the patient. (Beneficence)
- It may be the least restrictive option for patients to avoid hospital admission.

- Evidence suggests that they can improve under a Community Treatment Order, which reduces the risk of relapse and improve outcomes for patients.
- Clinical studies have shown benefits. Patients themselves may choose a Community Treatment Order or families may prefer a Community Treatment Order.

Evidence against Community Treatment Order:

- Use of a Community Treatment Order may go against the recovery principles as it may be considered as a violation of human rights.
- A Community Treatment Order may undermine the autonomy of the patient, which is a key tenant of the recovery model.
- Community Treatment Orders can disrupt the therapeutic alliance, which is associated with good outcomes in mental illness.
- Despite a Community Treatment Order, patients may relapse.
- Community Treatment Order increases health service use and requires higher resource allocation.
- Studies have shown negative and conflicting results as there are independent RCTs that have shown no benefits.

Question: Jarvis, on meeting with you, asks what the long-term serious risks are with regards to being on paliperidone depot long-term. Explain your response to Jarvis. List and elaborate (4 marks).

Answer: (For the exam write in shorter sentences; avoid repetition e.g paliperidone being a dopamine blocker..)

Extrapyramidal side-effects:

Paliperidone is a dopamine blocker and longer-term use of a dopamine blocker can increase the risk of tardive dyskinesia ie. abnormal, involuntary movements, mainly orobuccal dyskinesic movements. This can be disfiguring and can affect self-esteem.

Paliperidone long-term use can be associated with extrapyramidal side-effects, such as **Parkinsonian side-effects, tremors, bradykinesia, akathisia and subjective restlessness that can impact on mood and sleep.**

Prolactin:

Paliperidone, being a strong dopamine blocker and passing through the blood/brain barrier can increase prolactin. Prolactin can result in galactorrhoea, gynecomastia, sexual dysfunction, decreased libido and from a longer-term perspective can increase the risk of osteoporosis.

Can reduce testosterone which is associated with depression, anxiety and poor cardiovascular health.

Supervision principles and practical aspects of working with interpreters

You are a consultant psychiatrist working in the mental health service. Sarah, who is a stage 2 trainee, has been assigned to work with you. She contacts you with regards to arranging supervision and you set-up a time to meet with her.

Question: What principles of supervision, as per RANZCP guidelines, would you use in this instance?

Answer:

- Provide a collaborative goal-setting approach in a safe environment for issues to be raised, reflective practice and constructive learning.
- Regular, scheduled meetings of four hours per week over 40 weeks, at least one hour per week should include supervision direct one-to-one clinical work.
- Assist with the passing of the RANZCP exams. Assist with self-guided learning.
- Monitor and observe trainee's interaction with patients, peers and other medical staff.
- Discuss the performance with the director of training, if required. Discuss adequate strategies for personalised feedback.
- Strategies to overcome any weaknesses or challenges.
- Ensure availability to participate in workplace-based assessments and completion of end of term training reports.
- Feedback can be formative and summative. Summative feedback is often provided by RANZCP, subsequent to exams.

What are the principles of effective feedback?

Answer:

Feedback can be summative or formative

- The goal of formative assessment is to *monitor student learning* to provide ongoing feedback to students to improve their learning.
- The goal of summative assessment is to *evaluate student learning* at the end of a unit or rotation by comparing it against some standard or benchmark. (e.g WBA, Exams)

Feedback can be given both one-on-one or in a group setting. If feedback is meant to be one-on-one (individualised to the person), then provide a private setting.

Flag with the trainee that the feedback is a **formative** feedback. Base feedback on behaviours that the supervisor has directly observed.

Aim to develop a climate of trust.

Feedback should never be personally humiliating or delivered in an angry, threatening or disrespectful manner.

Provide an environment where the trainee can ask questions about how they can improve. Frank and direct feedback but respectful and constructive.

Start with student strengths, followed by feedback. Finish on a positive note with an enhancing statement. (Sandwich method)

Give direction to the trainee and invite comments about one's personal feedback.

Sarah informs you that you have an upcoming assessment of Fatima, a 39-year-old Iranian refugee, who is the mother of two children, where she wants to use a workplace-based assessment for the stage 2 EPA (entrusted professional activity) on CALD - Working with Culturally and Linguistically Diverse Population. She tells you the meeting is scheduled with an interpreter.

Question:

What are the principles of using an interpreter?

Answer:

Three phases of interaction: pre-interaction information, the actual interview and post-interview discussion

Pre-interaction information:

- Ensure that the interpreter is certified and registered.
- Obtain consent from the patient about involving an interpreter.
- Ensure appropriate language.
- Ensure confidentiality arrangements (that the interpreter isn't from a small community that can result in a breach of her confidentiality).

- Pre-interaction information and briefing:
Meet with the interpreter to provide information on what one is trying to achieve with regards to the examination
- Meet with the interpreter before the interview to explain the purpose or goal of the interview.

Interview:

- Speak slowly and clearly.
Use simple, layman's terms, where possible.
Speak to the patient, not to the interpreter
Clarify any confusing responses immediately.
Ask for verbatim translation if the translation is not clear. Avoid taking notes.
Concentrate on non-verbal behaviours of the patient.

Post-interview discussion:

- Meet with the interpreter afterwards for feedback.
- Interpreter's opinion on the normality of the conversation, speed of conversation and any aspects that would not be considered culturally appropriate or deviate from cultural norms

Sarah approaches you with an ECG of Fatima with concerns about the QTc reported as 450 ms. Fatima is prescribed an antipsychotic medication. What advice would you provide to Sarah?

Answer:

1. See the patient: Clinical state, pulse, temperature, blood pressure and clinical symptoms such as breathlessness or chest pain. If present, consider urgent repeat ECG and involvement of a cardiologist.

2. Previous baseline ECG and QTc. Has there been an increase in the QTC ie. a QTc prolongation? An increase of > 60 msec from baseline is cause for concern

3. Rule out organic factors that can increase the QTc interval:

- Renal dysfunction, liver dysfunction, electrolyte abnormalities, calcium, potassium, sodium, phosphate abnormalities and thyroid dysfunction.
- Advise that 450 ms is considered normal in women and the upper limit is 470 ms.

- > 500 msec QTc interval increases the risk of Torsades de Pointes by 2-3 times.
- If QTc has been prolonged from previous baseline ECG, identify any additional medications that have been prescribed that could result in drug interactions, such as antidepressants (citalopram, escitalopram), anti-emetics, antibiotics or any medications that may increase the levels of antipsychotics such as CYP2D6, CYP1A2 or CYP3A4 inhibitors.

4. Options for treatment include if QTc is significantly prolonged and intervention required:

- Involve a cardiologist.
- **Reduce** the dose of the antipsychotic and repeat an ECG.
- **Switch** to an antipsychotic medication that has a reduced effect on QTc such as lurasidone, aripiprazole or brexpiprazole.
- **Continue with close monitoring**
- Correct any organic factors.
- Address any other medications that might result in prolonged QTc.

Obsessive Compulsive Disorder

You are a junior consultant psychiatrist in the community mental health clinic. You are referred by a GP for assessment, James, a 39-year-old unemployed electrician living with his parents. James has had a diagnosis of obsessive-compulsive disorder over the last 20 years. He hardly ever leaves the house these days and relies on his parents for shopping, cooking, etc. He has been treated with medications in the past but he has self-ceased all medications for fear of side-effects. He has been linked in with a clinic in the past but disengaged after a few months.

Question:

What do you think are some of the possible reasons for James not engaging with the clinic in the past? List and elaborate (10 marks).

Answer:

This can be divided into patient factors, treatment factors and illness factors

Patient factors:

- Lack of belief in mental health treatments
- Stigma from the family
- Negative attitudes to medications
- Alternate beliefs in treatments eg. traditional healing, herbal medications, complementary therapies
- Sick role, abnormal illness behaviour, dependent traits

Illness factors:

- Comorbid agoraphobia
- Severe anxiety
- Social anxiety
- Comorbid depression resulting in motivation, Nihilism
- Hypochondriacal symptomology comorbid, resulting in fear of side-effects
- Dependant traits

- Abnormal illness behaviour
- Schizo-obsessive diagnosis with auditory hallucinations or delusions or fear of harm from medications, team, parents.
- Symptoms of obsessive-compulsive disorder that prevent him from leaving the house, such as beliefs that parents could be hurt if he leaves the house.

Treatment factors:

- Side-effects of medication.
- Cost of medication.
- Difficulty taking medication due to complexity of regime or cognitive deficits.
- Non-effectiveness of medication. The medication may not have been prescribed at the appropriate dose or for the appropriate duration.

Clinic factors: (under treatment or separate)

- Poor therapeutic relationship with treatment providers.
- Distance to travel to attend appointments

Following assessment, you diagnose him with obsessive-compulsive disorder without any other psychiatric or medical comorbidities or substance use disorders. James tells you that he is struggling with his OCD with compulsion/rituals, having to check and lock everything in the house. He says he is frightened to leave the house for fear that he might have left the stove or some appliances on and is fearful that his parents could be hurt.

He agrees to engage in treatment with you as long as he is not admitted to hospital. You decide to provide psychological therapy (CBT). How would you go about planning and implementing CBT in James?

Answer:

The CBT techniques in obsessive-compulsive disorder that are evidence-based are **Exposure- Response Prevention**.

The three phases of CBT include

Stage One:

Assessment, psychoeducation about the disorder and treatment planning:

- Developing a formulation about the anxiety disorder
- providing psychoeducation about obsessive-compulsive disorder ie. obsessions and compulsions
- the principle of habituation and extinction, the principles that maintain the anxiety
- triggers, fears and rituals
- address factors that facilitate or hinder therapy.

Stage Two:

- Reduce physical symptoms through relaxation, exercise, exposure to real-life situations in a hierarchical manner.
- Start with the least anxiety-provoking situation until the patient gains confidence and is able to prevent the compulsion.
- Exposure can be in vivo or imaginal exposure; 20 sessions, 60-75-minute sessions each.
- Encouraging patient to face fears; extinction of fear occurs to repeated exposure

Safety response inhibition:

- Patient restricts anxiety-reducing behaviours e.g compulsions that maintain the anxiety cycle. Restriction of these behaviours decreases negative reinforcement.
- Coping with anxiety without using anxiety-reducing behaviours enhances self- efficacy.
- **Relinquishing safety signals:**
Patient relinquishes safety signals such as compulsions and patients learn adaptive self-efficacy.

Cognitive processing:

- Increasing self-confidence
- Discuss what is learnt from the rituals and provide homework for practice outside the clinic setting.

Stage Three:

- Relapse prevention using the above techniques to maintain remission from the condition.

What are the pharmacological recommendations for treatment of OCD in James?

Answer:

1. First-line treatments are high-dose SSRIs.

- High doses of SSRIs are more effective than lower doses eg. citalopram 80 mg per day, escitalopram 40 mg per day; monitor QTc interval.
- SSRIs should be prescribed for at least 12 weeks to determine responsiveness.

2. If there is no response, consider second-line treatments, which include **clomipramine** started at 25 mg per day with a target dose of 250 mg a day; recommended monitoring for anticholinergic side-effects.

3. If non-response, other alternatives include **augmentation with antipsychotics** eg. aripiprazole 5-10 mg per day, olanzapine 5-10 mg per day.

4. **Lamotrigine, memantine, topiramate, NAC** 2400-3000 mg per day.

5. Other evidence includes **5-HT₃ antagonists** such as ondansetron 2-4 mg b.d, granisetron 1 mg b.d, **nutraceuticals** include Myo-inositol, 5-HT milk thistle.

6. **Treat comorbidities** such as depression, psychosis, comorbid anxiety disorders.

Governance and Leadership

You are a junior consultant psychiatrist working in a regional area. There has been an electroconvulsive therapy (ECT) adverse event (critical incident) involving ECT for a patient.

A First-year student asks you to explain what a root cause analysis is.

List/explain/describe (8 marks)

Answer

A root cause analysis approach is an analysis used to identify the underlying causes of system failure.

Clinical risk managers and other healthcare professionals use root cause analysis to help find answers to questions posed by serious incidents.

It is an important part of clinical governance and aims to ensure that there is accountability and responsibility for the services that are provided and is underpinned by continuous quality improvement, thus any system deficiencies that are identified via the root cause analysis and implemented.

It is performed for high-risk, high-impact events.

Identify the critical incident to be reviewed.

Arrange a committee that involves clinical risk personnel to evaluate it, which consists of a consumer, possibly a carer, team member directly or indirectly involved in the incident, consultant psychiatrist or clinical director.

Individuals that are chosen for the team are not individuals necessarily that have directly been involved with the incident. The principle is that the focus is on systems and processes, not the individual. This is done to avoid scapegoating

It takes into account a problem-solving methodology using analytical methods. The principles are TFEI:

- Thoroughness
- Fair
- Efficient
- Independent review

The steps include:

- Verify the incident
- Commission a Root Cause Analysis Team Map a timeline
- Identify critical events
- Analyse the cause and effect chart
- Identify root causes
- Support root causes with the evidence
- Find solutions
- Provide recommendations

Teaching sessions to stage 2 trainees on the background, prescription and administration of ECT. Outline what you would do and how you would do it. (14 marks)

Answer: (Note that this answer is overinclusive; please be mindful of time - you are not expected to write all details)

Electroconvulsive therapy is an evidence-based therapy for a number of psychiatric conditions.

1. Indications: (RANZCP Guidelines) are:

- Treatment-resistant major depressive disorder
- Major depressive disorder with psychotic features
- Major depression;melancholic features (treatment-resistant)
- Major depression; peripartum onset
- Bipolar disorder(resistant mania and depression)
- Depressed patients who have not responded to adequate trials of medication and psychotherapy
- Need for rapid clinical improvement
- First-line treatment for severe depression where there is an inadequate oral intake, a high suicide risk
- High level of patient distress

There is also some evidence of ECT in treatment-resistant schizophrenia

2. Background:

Efficacy and response rate

- The response rate for unilateral ECT with melancholic and psychotic depression who have failed one or more medications trials are as high as 48%.
- In psychotic depression, the remission rates may be up to 95% and those with non-psychotic depression, 83%.
- ECT can also reduce the suicidal thoughts, actions or gestures in up to 70% after nine sessions.

3. Key considerations in ECT administration and prescription:

- Dosing
- Electrode placement
- Pulsewidth
- Session frequency
- Concomitant medication
- Anaesthetic approach

Pre-ECT assessment and informed consent: (You don't need to mention this in detail - a separate question can be asked hence the following points are covered))

- Explanation of treatment
- Methodology and process
- Adverse events and side effects
- Risk of transient anterograde and retrograde cognitive impairment
- Irreversible retrograde memory loss particularly if bi-temporal techniques are utilised.
- The patient should be informed what to be expected before, during and after administration.
- Involve families and carers in the process by taking into account advance directives.
- Make aware that second opinions can be obtained.
- The patient should be reminded they can withdraw consent at any time during the course of the ECT.
- Clear documentation and notes.

4. Dosing:

Establish the seizure threshold (The seizure threshold at the lowest electrical dose that elicits seizure activity as seen on EEG or visible motor movement).

The methods to establish seizure threshold: empirical dose titration, age-related or half-age method.

5. Electrode placement:

- Bitemporal
- Bifrontal
- Right unilateral

Pulse width:

- ECT dosing up to 6x threshold for unilateral ECT while 1.5-2 x seizure threshold for bilateral ECT.

Session frequency:

- The number of ECT treatments required by the patient is guided by the patient's progress and clinical improvement.
- Courses of ECT consist of 6-12 treatments. Ultra-brief pulse method also present (0.25- 0.3 ms).

6. Anaesthetic management:

- Pre-ECT evaluation by the anaesthetist.
- Ensure the seizure threshold is not excessively elevated by drugs or dosage used.
- Commonly used agents are thiopentone and Propofol.
- Muscle relaxants, such as rocuronium or vecuronium to administer before suxamethonium to minimise suxamethonium-induced myalgia.

Monitoring:

Cognitive Assessment (MoCA) or Brief ECT Cognitive Screen and neuropsychological assessment if indicated.

7. Post-ECT:

- EEG parameters of an adequate seizure (e.g adequate postictal suppression)
- Evaluation in an ECT suite for any delirium, cognitive impairment.
- Techniques to reduce cognitive side effects include bilateral ECT ultra-brief pulse and Focal electrically administered seizure therapy (FEAST).

Adult ADHD prescribing is higher than your peers in the region. How would you respond and manage? Your clinical director hands you an evaluation showing that your practice was prescribing psychostimulants

vastly different to those of your peers. How would you go about addressing this?

Answer:

This question points to quality improvement or an audit of one's own practice.

Steps in the audit:

1. Identifying the process to be reviewed, which is ADHD prescribing compared to peers.

2. Establishing a gold-standard.

- Literature review
- speaking to colleagues
- peer review groups
- Identifying psychostimulant prescription related to patient presentations.

3. Identifying the discrepancy.

4. Identifying the causes for the discrepancy.

- Patient factors
- Clinical factors
- System factors

Patient factors:

- Higher presentation of ADHD.
- A greater number of patients with substance abuse presentations, which is linked to ADHD presentations.
- Better access for patients that assists in an early pickup of ADHD.
- Other patient factors may include greater awareness in the community about ADHD.
- Good mental health literacy about ADHD in community

Clinical factors:

- Other psychiatrists or other colleagues may be hesitant in prescribing psychostimulants
- Lack of training or inadequate training about the diagnosis of ADHD or prescribing psychostimulants.

- A negative attitude towards prescribing psychostimulants or diagnosis of ADHD.

System factors:

- Lack of private psychiatrists or psychiatrists to prescribed psychostimulants in the community to treat ADHD
- The possibility that word-of-mouth with patients presenting with ADHD to the clinician.

Self factors:

- The possibility that prescribing may be inadequate, therefore, seek peer review supervision and an independent review

5. Implement solutions to manage the discrepancy.

- Independent peer review
- Seeking advice about appropriate training in ADHD and prescribing of stimulants
- Attending conferences, upskilling, CPD.
- Grand round discussion or the consultant peer review group to obtain an open and transparent understanding of prescribing practices with regards to psychostimulants and reasons for this.
- A qualitative analysis. Answering **why** prescribing is greater rather than the statistics.
- **Seeking advice from medico-legal insurance** and ensuring good documentation, diagnosis of ADHD, obtaining collateral information when making the diagnosis and prescribing according to the States and Territories Guidelines.

6. Re-Audit in 6 mths-1 year

Clozapine and Antipsychotic Switching

A 45-year-old gentleman with schizophrenia working as a gardener recently relocated from another service. You have access to his previous information.

He is able to make decisions for himself. He is stable on paroxetine 400mg daily for the last 15 years. The new case manager tells you the hand over from previous community mental health team has been adequate and all relevant documents have been sent across.

Joe wants to change his anti-psychotic because he is struggling with side effects of tiredness, sleeping 12 hours a day, his head feels foggy and he can only work two days a week (as a gardener) due to these symptoms. He is compliant with medications but on the odd occasion that he forgets his clozapine, he feels brighter. What information would you like to find out before you consider Joe's request for medication change?

1. Current clozapine level: (350-600 ng/ml)

- Clozapine levels may be elevated due to a range of interactions.
- Identify smoking habits
- caffeine habits
- Additional medication that may increase the levels of clozapine or recent infections resulting in antibiotics, such as ciprofloxacin or erythromycin, which are CYP1A2 inhibitors.
- Smoking recently being stopped, resulting in an increase in clozapine, or the use of antidepressants, such as fluvoxamine.

2. Past psychiatric history

- Past history of mental illness
- nature of illness
- severity of illness
- risks when unwell
- risk of harm to self and others
- compliance with medication
- Previous side effects from clozapine and how they were managed
- Is this new or has he previously complained of sedation?

3. Past insight and compliance with medication

4. Reasons for sedation and tiredness

Organic factors, such as hypothyroidism, obstructive sleep apnoea.

Other medication resulting in over-sedation, such as quetiapine combination, and substance-induced (benzodiazepine; alcohol use).

Use of sleep medication in conjunction with clozapine.

Extrapyramidal side effects from the use of clozapine

Alternate antipsychotic prescribed in conjunction with clozapine resulting in blunting tiredness, and fatigue.

Current Mental state Exam:

- Presence of delusions or hallucinations resulting in sleep difficulties or agitation
- Presence of depression resulting in tiredness and fatigue, memory issues
- Use of cannabis resulting in an amotivational-type syndrome

You decide to go ahead and change Joe's antipsychotic, what strategies can you use to safely transition him from clozapine to another antipsychotic?

Risk/benefit analysis

- Clozapine has strong antihistaminergic and strong anticholinergic properties. Therefore, sudden cessation of clozapine can result in antihistaminergic and anticholinergic rebound that can present with severe rebound insomnia, severe rebound affective symptoms and severe rebound psychosis. In addition, anticholinergic delirium is also a possibility.
- Therefore, the transition to an alternate antipsychotic should take these into consideration
- From an evidence-based perspective, clozapine is evidence-based in treatment-resistant schizophrenia.
- Evidence suggests that olanzapine may be appropriate, but newer antipsychotics that are metabolically friendly may be considered, such as lurasidone, brexpiprazole or aripiprazole.
- Abrupt cessation of clozapine carries a high risk of rebound psychosis and rebound anticholinergic symptoms. If rebound psychosis occurs, it can be severe and accompanied by physical and autonomic symptoms that resemble neuroleptic malignant syndrome.

- If possible, clozapine should be stopped very slowly over 6 weeks. Very high-risk patients should stop clozapine over 3–6 months.
- If the patient is changed from clozapine to olanzapine, olanzapine also has antihistaminergic and anticholinergic properties and transition can reduce antihistaminergic or anticholinergic rebound.
- If clozapine is changed to brexpiprazole, lurasidone or aripiprazole, these antipsychotics lack the significant anticholinergic and antihistaminergic property and pose a risk of relapse, therefore, clozapine should be reduced to a low dose whilst the patient is stabilised on an adequate dose of lurasidone or a non-antihistaminergic/antipsychotic and clozapine should be stopped after an extended period of time to a gradual reduction to prevent antihistaminergic/anticholinergic rebound.
- To prevent the risk of relapse, an inpatient admission may be appropriate
- If inpatient admission cannot be implemented, an assertive outreach approach or regular post-follow up appointments of case manager would be appropriate.
- Alternatively, psychoeducation can be provided about risks and benefits, and early relapse signs.
- The involvement of family to detect early relapse signs. The principle is that the transition should be done extremely gradually, which may be over a period of six months to even over a year.

A medical student asks you to explain what the withdrawal and discontinuation syndromes one might experience from stopping an antipsychotic. How would you answer this question? (4 marks)

- The withdrawal symptoms and discontinuation symptoms with antipsychotics are mainly due to dopaminergic, antihistaminergic and anticholinergic rebound. For antipsychotic medication such as olanzapine, clozapine and quetiapine which have strong anticholinergic and antihistaminic antagonism, discontinuation may result in rebound insomnia, affective symptoms and psychotic symptoms and a relapse of the illness.
- Reduction, therefore, should be done in a graded manner.
- Refer to a switching tool. [NPS switching Tool](#).
- If changing to an antipsychotic with an antihistaminergic property, then a direct cross-titration can be considered as this reduces the risk of antihistaminergic and anticholinergic rebound. Close monitoring should be in place when considering changeover. The appropriate choices of antipsychotic should be considered when transitioned.

Withdrawal and discontinuation syndromes:

1. Cholinergic syndrome

- **Nausea, vomiting, headache**
- **restlessness, anxiety, insomnia**
- **fatigue, malaise, myalgia, diaphoresis, rhinitis, paraesthesia, loose bowels**

2. Dopaminergic syndrome

Withdrawal dyskinesia, akathisia, dystonia, tardive dyskinesia

3. Rebound psychosis (Dopaminergic)

Psychosis above pre-treatment levels, delusions, hallucinations, catatonia

Management of Acute Aggression

Question 1: **[Please note these are questions from memory. So there may be some ambiguity. We are working to cover the main principles. The answers have focused on broad domains. Ensure you add specific aspects in those domains depending on the number of marks.]**

You are an on-call psychiatrist in a large hospital. Your registrar calls to get advice on a case. A male has been brought into ED after his wife called the police reporting she and her child are unsafe from him. He is not known to mental health services. He is well known to police who have had contact with him on multiple occasions and he has a history of amphetamine possession. [Aggressive man brought in by police on Amphetamine after his wife and kids felt threatened and contacted police]. He received 10mg IV Droperidol twice and IM Midazolam for sedation and needed physical restraint. He is unresponsive after that.

5.1 List immediate management (4 marks)

1. **CPR and medical emergency call** for resuscitation if the patient is unresponsive.

2. **Monitoring of vital signs and level of sedation**

- IM midazolam can be associated with unpredicted oversedation, oxygen desaturation, airway obstruction requiring intervention
- Droperidol adverse effects include hypotension, respiratory depression (especially if administered with benzodiazepines), extrapyramidal side effects (laryngeal dystonia) and QT prolongation leading to torsades de pointes.

3. **Setting of treatment:** Sedation of the patient should occur in an area where resuscitation equipment and appropriate monitoring is immediately available and wherever possible should be undertaken in a resuscitation room

4. **Collateral from family:** Risk assessment - safety of the child and safety of wife. Illness history

5. **Safety when the patient is awake** - inform security. 1-1 nurse special. Separate room. IV line intact with monitoring of vitals if further tranquillisation is required. Close monitoring with psychiatric input to differentiate between aggression due to illness and differentiating delirium.

If the question is on managing acute aggression in the context of psychosis/methamphetamine intoxication or use, then please see this step-wise protocol for management.

Question 2:

The nurse says he is awake and physically stable and requests a psychiatric assessment

What will you ask the registrar to check? (6 marks)

1. Safety and suitability for assessment -

- Safe space
- 1-1 nursing
- Separate room
- Security informed
- Duress alarm
- Assessment with colleague

2. Risk assessment:

Absconding, self-harm, harm to others, access to objects that can be used as weapons.

Safety of child and wife - Is child protection involved?

3. Assessment for medical causes of behavioural disturbances

- General medical condition: acute delirium, head trauma, encephalitis, meningitis
- Infection, infected needle marks, encephalopathy (particularly from liver or renal failure), metabolic derangement (e.g. hyponatraemia, hypocalcaemia, hypoglycemia), hypoxia, seizure (post-ictal)

4. Psychiatric assessment / Mental state examination

- Psychosis - agitation, responding to unseen stimuli, delusions
- Depression - depressed affect / suicidal ideation, guilt, worthlessness
- Mania - pressure of speech and flight of ideas, grandiose ideation

5. Assessment for Intoxication/withdrawal:

Amphetamines: tachycardia, sweating, tics, hallucinosis

Alcohol, hallucinogens, cannabis, synthetics, opioids, benzodiazepines

6. Psychosocial assessment -

AVO or intervention orders present due to violence

Question 3: Registrar asks you what the principles of management of safe practices in Acute severe behavioural Disturbances? (12 marks) - [main principles have been stated - please add info as needed in exam. much of it has been covered in other questions previously

1. Safety
2. Medical and psychiatric evaluation - as in question 2
3. Deescalation techniques:

Approach in a calm, confident and non-threatening manner, with a non-aggressive stance with arms relaxed.

Be empathic, non-judgemental and respectful. Listen to the patient's concerns.

Use a slow, clear and steady voice and don't raise your voice. If the patient raises their voice, pause and wait for an opening and allow the patient to vent some of their frustrations.

Getting trusted relatives or staff to talk to the patient may help. If the patient persists in directing their anger or suspicion directly at the clinician, it may be appropriate for you to ask another staff member to attempt de-escalation.

4. **Legal aspects** – Guardianship / MHA / Restraint and seclusion
5. **Management of acute behavioural disturbance** - The aim is to achieve an appropriate and safe level of sedation quickly with sufficient medication to manage acute behavioural disturbance and to facilitate an accurate assessment and appropriate management of the patient's underlying condition. The level of sedation should ensure that the patient is drowsy but rousable. The procedure is not intended to render the patient unconscious.
6. **Monitoring - setting**
7. **Debriefing:** The experience of sedation and/or restraint can be a distressing experience for the patient. A skilled staff member who is able to listen to the patient's experience as well as provide feedback and rationale for the intervention should be considered in the patient mental health care plan at a later stage. Consideration should also be

given to the emotional wellbeing of other patients and/or visitors who witness such events.

Psychotherapy

Question 1

Young IT clinician, not socialising, spends most of the time on the computer late at night, well-functioning but thinks that "I am a loner, I am a loser". His father died 3 years back, he enjoyed wood cutting with him and misses him. Isolating in past 3 years has gained weight.

You have diagnosed him with mild to moderate depression and he wants to do psychotherapy.

List and debate the MOST important psychotherapy options for him. (12 marks)

In this answer, it is important to follow a pattern

Psychotherapy modality / Principles or reasons for benefits or Evidence in depression / Linking psychotherapy modality to the patient clinical picture. **Not all of these modalities need to be mentioned . CBT has been discussed - please do this with other therapies that you choose.**

Cognitive behavioural therapy - Evidence-based treatment in mild to moderate depression. Depression is linked to negative dysfunctional assumptions of self - e.g low self-esteem, low self-worth, self-doubt. These affect emotions leading to low mood, feelings of worthlessness and hopelessness which in turn affect behaviour e.g avoidance behaviour. CBT addresses these core domains e.g Cognitive strategies include - thought substitution, challenging, reframing. Behavioural strategies to combat avoidance - behavioural activation. In this case, CBT could address negative cognitions e.g "I'm a loser, I'm a loner " Behavioural activation - reinstating wood cutting - exercise could also lead to weight loss improving self-esteem and may combat social isolation.

Interpersonal psychotherapy - Role transitions, interpersonal deficits, role disputes and grief.

Schema therapy- similar to CBT - core scheme addressed

ACT

Social skills

Grief therapy

Behavioural activation

Non-directive supportive therapy

Problem-solving therapy

He prefers face to face CBT

- List and explain what will you cover in homework with him. (9 marks)

Behavioural Activation:

Getting active, depressed patients out of bed or off the couch, and helping them resume normal activity

Monitoring automatic thoughts:

Encourage your patients to ask themselves, "What's going through my mind right now?" thought diary

Evaluating and responding to automatic thoughts

Help patients modify their inaccurate and dysfunctional thoughts and write down their new way of thinking. Patients will also learn to evaluate their own thinking and practice doing so between sessions. Thought diary

Problem-solving

Help patients devise solutions to their problems, which they will implement between sessions.

Behavioural skills

To effectively solve their problems, patients may need to learn new skills, which they will practice for homework.

Behavioural experiments

Patients may need to directly test the validity of automatic thoughts that seem distorted, such as "I'll feel better if I stay in bed".

Bibliotherapy

Providing reading material. Important concepts you are discussing in session can be greatly reinforced when patients read about them in black and white.

Preparing for the next session

The beginning part of each therapy session can be greatly sped up if patients think about what is important to tell you before entering your office.

He reported to the registrar that you are supervising that he does not want to do homework.

List and explain factors preventing him from doing homework. (7 marks) - Similar questions have been covered previously so please refer to those.

- Patient factors
- System factors
- Therapy factors
- Clinician factors

Perinatal Psychiatry and IVF

39 years old, single lady, on DSP, diagnosed with chronic Schizoaffective disorder, maintained on Clozapine 600mg and lithium, no substance use disorder, good compliance and engaging well with the last admission 3 years back. Her BMI is 35.

Does she want to have a baby via IVF?

2.1 What advice you will give her. (9 marks)

Key domains: please fill in necessary material relevant to each domain. focus on key issues.

Here the assumption is she is well. But if the question doesn't allude to that - current mental state exam and capacity to make decisions is a separate domain.

1. Sensitive discussion about desire to have baby.

Non-judgmental attitude, reasons to have a baby (personal goals), her understanding of risks of having baby and risk of illness and medication. Current partner. Is she sexually active? Is she on contraception?

Positive validation that she is planning in advance - shows good insight.

2. Risk-benefit analysis - similar to all perinatal psychiatry questions

Risk of medications on pregnancy/risk of illness relapse

3. Specific risk of IVF - as IVF includes hormonal treatments and can be a stressful period - the risk of relapse is to be monitored. close monitoring with IVF specialist. IVF specialists may require a report or letter regarding capacity and mental state to proceed with IVF.

4. Risk of illness relapse and impact on pregnancy and fetus

6. Risk of clozapine during pregnancy and monitoring levels

7. Risk of lithium during pregnancy and monitoring levels

8. Risk of clozapine during breastfeeding and monitoring levels - agranulocytosis - formula feeds, expressed breast milk, timing feeds

9. Risk of lithium during breastfeeding and monitoring levels

10. **Liasion:** Specialist perinatal service / IVF specialist / obstetrician / midwives / lactation consultant

She delivers a baby and 3 weeks postnatal, the midwife contacts you concerned about her that she believes that God was telling her that it's a devil's child and she is refusing to seek mental health assessment.

2.3 How will you respond to her concerns. (7 marks) - Question is a bit unclear regarding re: setting. Is it at home? Is it advice to midwife only? In exam questions are clearer. The answer will cover basic principles - modify according to the specific question.

1. Advise that she is displaying symptoms of postnatal psychosis and this is a **psychiatric emergency needing urgent assessment**. There are significant risks to the mother and baby. However medical risks also need to be ruled out - lithium toxicity with delirium/clozapine rebound/clozapine toxicity with anticholinergic symptoms.

2. **Therefore advise immediate assessment** - Where are the mother and baby? Police may need to be called if the patient is refusing engagement and lacks insight. Emergency psychiatric team to attend together.

3. **Collateral from partner or family** - Onset of symptoms. evidence of risks, medication compliance

4. **Child protection services** - Are they involved?

5. **Assessment** - Does the patient fulfil the criteria for admission under MHA?

Organic causes to be ruled out - sudden cessation of clozapine or lithium or toxicity

Infection - therefore medical evaluation should also be carried out.

6. **Setting of admission** - Mother baby unit with close monitoring or special if needed

Delusions vs Religious Beliefs

Marian is diagnosed with Squamous cell carcinoma and the surgeon wants to operate but he is refusing treatment.

He believes that god will cure him.

Question 1: How will you differentiate psychotic beliefs and religious beliefs / Describe the phenomenology of psychotic vs non-psychotic religious beliefs? (7marks)

A delusion is a false, unshakeable idea or belief, which is out of keeping with the patient's educational, cultural and social background; it is held with extraordinary conviction and subjective certainty

1. Delusion is held without any doubt, but religious belief is held with some doubts or at least an understanding that others could have doubts.
2. Delusions are concrete/religious views have a degree of abstraction - Religious beliefs are spiritual, abstract, not concrete – 'God within me' is not experienced as a tactile sensation
3. Religious beliefs are held with insight - it is understood that others may not share their beliefs.
4. For religious people, bizarre thoughts and actions do not occur in other areas of life, not connected with religion. Delusional beliefs will often have other delusions in other areas.
5. Delusional beliefs are often associated with functional and social impairment / religious beliefs less likely.

One of the ward nurses knows him from the church group. Registrar asks her to go and talk to his Church member/ family to come and talk to us. The nurse refuses and declines it.

What are the professional issues in the request made by the registrar to the nurse? (10marks)

1. **Autonomy** - Patient wishes and decisions are important. Has the patient expressed a wish to convey this to the nurse? If yes does the patient have the capacity to make an informed decision after understanding the risks and benefits of this request?
2. **Privacy and confidentiality** - speaking to the nurse without patient consent constitutes a breach of confidentiality

3. **Boundary violation** - Asking the nurse to convey this to church members is beyond the boundary of usual clinical care. Clear boundaries are essential between the clinical need for information in relation to treatment vs boundary violation.
4. **Beneficence:** Confidential information about patients illness conveyed to their parities - Is this in the patient's best interest?
5. **Non-maleficence** - Can disclosure of information cause harm? - ostracisation from church group/risk to reputation/lack of objectivity by professionals involved
6. **Registrar supervision:** Ethics / good practice/understanding of boundaries in care

Cultural Scenario

42-year-old married Indonesian woman lives with her husband and their 3-year-old child. She presented following an overdose of 10 tablets of 10mg amitriptyline. She felt hopeless that her physical health will not return to normal functioning.

When you saw her, she was adamant that she was no longer suicidal. She feels depressed, tired, lethargic, predominantly somatically focussed and not able to work as a tutor at university.

Her husband is *not happy* that she is not doing any work or looking after her son and this has caused conflict in their relationship.

You see her in the post-overdose clinic.

Husband tells you that he is not happy for her to be followed up by a psychiatrist.

Q1. List and explain possible causes of stigma in this situation

Husband related

- Culturally alternative explanatory models of illness
- Negative attitudes to western models of care
- Belief in alternative or culturally alternative treatment methods e.g. traditional healing, herbal treatments
- Strong power imbalance within a male dominated culture
- Religious beliefs

Treatment-related

- Lack of trust in services
- Previous negative experiences with mental health services
- Distrust of authority
- Belief treatment will change his wife's personality or make the situation worse – over-sedated or unable to look after the child

Illness related

- Belief that mental illness is a sign of weakness
- Mental illness linked to the patient's own will – i.e. Wife being 'lazy'

System-related

- Cost of treatment
- Time away from children
- Lack of gender matching or cultural/ gender sensitivity

42-year-old married Indonesian woman, lives with husband and their 3-year-old child. She presented following an overdose of 10 tablets of 10mg amitriptyline. She felt hopeless that her physical health will not return to normal functioning.

When you saw her, she was adamant that she was no longer suicidal. She feels depressed, tired, lethargic, predominantly somatically focussed and not able to work as a tutor at university.

Her husband is *not happy* that she is not doing any work or looking after her son and this has caused conflict in their relationship.
You see her in the post-overdose clinic.

Q2: GP has thoroughly examined and completed investigations and there's no identifiable cause to explain her somatic symptoms and thinks she is depressed.

List and explain the possible differential diagnosis

Please note the below points are guidance - please use specific aspects from the vignette to justify the diagnosis.

Major Depressive Disorder :

- MDD in some cultural groups presents with predominant somatic symptoms

Adjustment disorder with depressed mood – in the context of stressors, marital conflict, looking after a 3-year-old

Relationship difficulties – marital conflict – exposure to criticality contributing to low self-esteem and low mood

Untreated post-natal depression

Chronic fatigue syndrome with somatic symptoms

Grief reaction in the context of losses post-migration and also recent loss of job due to illness – feelings of hopelessness

Trauma-related mood symptoms – unrecognised domestic violence or emotional abuse

Abnormal illness behaviour or sick role with secondary gain

Chronic pain syndrome

She agreed to engage with psychological services. Unfortunately, only attended 2 sessions and then disengaged.

Q3:

Your consultant asks you what are the things that you will want to do before closing her case OR

List and justify the issues around non-engagement that you would discuss with your supervisor / Exploration of factors impacting psychotherapy

Personal factors

- Cultural alternate explanatory models of illness
- Language barriers
- Stigma toward mental illness
- Negative attitudes to western models of mental illness
- Belief in alternative or culturally different mental health treatments
- Presence of mental illness with symptoms impeding engagement in psychotherapy
- Appeasing husband

Psychotherapy factors:

- Ineffective psychotherapy
- Poor therapeutic relationship
- Cultural and language sensitivity not taken into account
- Gender sensitivity barriers
- Side effects of psychotherapy

System factors:

- Travel
- Cost
- Unable to leave her child to attend psychotherapy
- Explore risk and refer to acute mental health services if risk identified (includes child protection services if risk to the child identified or social worker if domestic violence identified)

- Documentation of reasons for discharge from psychotherapy in notes
- Discharge Letter sent to patient and GP
- Recommendations to GP made about further treatment and pathways if the patient wishes to reengage.

MEQ – Course Revision Part 2

Table of Contents

<i>Factors Increasing Risk of Sexual Recidivism</i>	<i>2</i>
Sexual violence risk assessment instruments	3
Items of the Sexual Violence Risk-20 (SVR-20).....	3
<i>Practical Aspects of Assessment in Sexual Offending</i>	<i>5</i>
<i>Assessment of Sexual Deviance</i>	<i>6</i>
<i>Management of Sexual Offending.....</i>	<i>7</i>
<i>Encopresis.....</i>	<i>8</i>
<i>Adult ADHD</i>	<i>11</i>
<i>Depression in Pregnancy.....</i>	<i>16</i>
<i>Management of Pain and Addiction on Ward.....</i>	<i>18</i>
<i>Indigenous Psychiatry.....</i>	<i>22</i>
<i>CTO and Ethics.....</i>	<i>25</i>

Factors Increasing Risk of Sexual Recidivism

1. Historical Factors

- Sexual violence history
- Non-sexual violence history
- Childhood problems
- Young age
- Employment problems
- Relationship problems
- Substance misuse
- Response to supervision
- Social influences

2. Stable Dynamic

a. Intimacy deficits:

- Capacity for relationship stability
- Emotional identification with children
- Hostility towards women
- General social rejection
- Lack of concern for others

b. Pro-offending attitudes:

- Rape supportive attitudes
- Child molester supportive attitudes
- Sexual entitlement

c. General self-regulation:

- Impulsive
- Poor problem-solving
- Negative emotionality

d. Sexual self-regulation:

- Sexual preoccupation

- Sex as coping
- Deviant sexual preference
- Cooperation with supervision

3. Acute Dynamic

Triggers

- Increase in substance misuse
- Deteriorating relationships
- Increased contact with pro-offending peers
- Problems getting or keeping work
- Increased access to potential victims
- Situation/state
- Increased sexual preoccupation
- Mood/mental state
- Creating opportunities or making plans to offend
- Using drugs/alcohol to deal with negative emotions
- Isolation
- Increasingly chaotic lifestyle

Sexual violence risk assessment instruments

Actuarial

- Risk Matrix 2000 (Thornton, 2007)
- Static-99 (Hanson, 1999)
- Sex Offender Risk Appraisal Guide (SORAG)

Dynamic

- Stable and Acute 2007 (Hanson, 2007)
- Structured Assessment of Risk and Needs (Webster 2006)
- Violence Risk Scheme – Sex Offender version (Wong 2010)

Structured professional judgement

- Sexual Violence Risk-20 (Boer 1997)
- Risk for Sexual Violence Protocol (RSVP; Hart 2003)

Items of the Sexual Violence Risk-20 (SVR-20)

1. Psychosocial adjustment

- Sexual deviation

- Victim of child abuse
- Psychopathy
- Major mental illness
- Substance use problems
- Suicidal/homicidal ideation
- Relationship problems
- Employment problems
- Past non-sexual violent offences
- Past violent offences
- Past supervision failure

2. Sexual offences

- High-density sex offences
- Multiple sex offence types
- Physical harm to victim(s) in sex offences
- Uses weapons or threats of death in sex offences
- Escalation in frequency or severity of sex offences
- Extreme minimisation or denial of sex offences
- Attitudes that support or condone sex offences

3. Future plans

- Lacks realistic plans
- Negative attitude towards intervention

Practical Aspects of Assessment in Sexual Offending

The Interview:

Explain the process and allow times as the individual may be guarded, embarrassed, reticent or wary.

Sexual history

- **Sexual knowledge and sources of information**
- **Sexual attitudes to self and others** (including attitudes permissive of sexual behaviour with children, rape supportive beliefs and general attitudes to sex), sexual self-esteem and sexual narcissism
- **Sexual development:** age at onset and development of puberty, sexual interests, masturbation, dating, sexual intercourse
- **Relationships history:** age of self and partners, gender of partners, duration, number of relationships (including number of age-appropriate sexual partners), quality of relationships, problems, fidelity and abuse
- **Fantasy:** content, use and development
- **Orientation:** sexual interest in males, females, children
- **Sex drive:** strength of libido, sexual preoccupation (including frequency of sexual outlets over a period of time), hypersexual arousal
- **Sexual dysfunction:** impotence, premature ejaculation, delayed ejaculation
- **Experience:** range of sexual behaviours and specific questions about paraphilias
- **Current sexual practices:** mood, material used, thoughts or visual images, conditions for arousal during sex and masturbation, frequency and nature of sexual outlets
- If assessing **risk then include assessment of risk factors** (Historical, Stable dynamic and Acute dynamic)

Reference: (Brockman and Bluglass, 1997)

Assessment of Sexual Deviance

Objective methods:

- Penile plethysmography
- Viewing time assessment
- Screening Scale for Pedophilic Interests, SSPI;
- Sexual Sadism Scale, SSS
- Self report scales: Multiphasic Sex Inventory, MSI
- Risk assessment scales : SORAG / SVR-20/ RSVP
- Personality assessment
- Psychometric testing

Management of Sexual Offending

1. Monitoring and Supervision

2. Psychological

Group cognitive-behavioural therapy (CBT)

The Risk, Needs, Responsivity approach (Andrews 2007)

The Good Lives model (Ward 2006)

3. Pharmacological

Antiliberals (e.g. gonadotrophin-releasing hormone agonists, cyproterone acetate)

Selective serotonin reuptake inhibitors (SSRIs)

Reference: Russell, K., & Darjee, R. (2013). Practical assessment and management of risk in sexual offenders. Advances in Psychiatric Treatment, 19(1), 56-66.

Encopresis

You have been referred to see a 6 year old boy, Sam, by his Paediatrician for encopresis. He lives with his mother who is a single mother. The Paediatrician has ruled out any organic issues and has started him on laxatives and a start chart which has not worked.

Outline your assessment of his encopresis.

1. Multimodal Approach involving Mother, Sam, General practitioner, paediatrician, school and school psychologist of present. It may be helpful to involve father if possible.
2. Sensitive issue and a non-judgemental approach to be adopted so that both mother and Sam do not feel blamed.
3. **Risk:** high index of suspicion for abuse should be kept at all times which may not be possible to decipher in the first interview. Longitudinal assessments may be required. Observe for any bruises or behaviour that may suggest abuse. May need expertise of psychologist trained in Child health.
4. **Contact paediatrician** to obtain notes for organic evaluation: Bowel and electrolyte abnormalities, neurological issues with sphincter weakness, Hirschprung's disease, Hypothyroidism. Examination of gait, spina bifida, muscle tone and strength. MRI , Ultrasound carried out. Abdominal examination for masses. Basic examination to be carried out if gaps present.
5. **Interview approach:** Interview Sam independently and with mother. Interview mother separately as well. Observe parent-child interactions and identify type of attachment (secure, ambivalent, disorganised)
6. **Evaluation of encopresis:** Age on onset (> 5 years), location (school or home or pervasive), Have siblings had it?, Toilet training methods, response to encopresis: Is punishment used or positive reinforcement? How is mother coping. Is there any violence towards Sam? Height and Weight for Sam.
7. **Developmental delay,** temperament, illness and trauma exposure. Separation anxiety or any separations when young?

8. **Psychosocial issues:** Domestic violence, financial stressors, dysfunctional relationship with father.

9. **Behavioural Analysis:** Antecedents, behaviours and consequences around encopresis

10. Causes for start chart not working: Is it being used consistently? Is positive reinforcement being used? Are the laxatives been administered regularly?

11. **Collateral from school:** Academic performance, any suspicion of abuse or bullying? Conduct disorder or any parenting concerns.

Sam has not seen his father for some time. His mother has depression and is on antidepressants. His mother said that she is tired of bribing him to behave. When you see Sam, you notice that he has a bruise on his arm. Outline your immediate management plan.

1. **Immediate assessment in the emergency department** to rule out serious injury e.g. fracture, internal bleeding etc. Full physical examination including ruling out stigmata of abuse or neglect.

2. **Consideration of possibility of abuse:** Involve a Social worker or child health professional in the assessment as it is a delicate situation and special expertise is required. Child protection services to be informed in case of suspicion of child abuse or neglect is identified (Statutory obligation)

3. **Interview approach.** Child will need to be interviewed separately and with mother. Mother will also need to be interviewed separately.

4. Adopt a non-judgmental stance and attempt to establish therapeutic alliance with mother. Enquire about her current illness, current coping strategies and her supports. Is she able to meet Sam's needs? Explore what is meant by bribing him? Does she use punishment as a strategy. Has she been violent towards him? What is Sam's relationship with the father. Has his father been violent towards him?

5. Involve specialist child and adolescent services to provide follow up in an out reach model.

6. Seek advice from Child and Adolescent Psychiatrist / Medico-legal insurance / AMA if any legal dilemmas are posed

Principles in responding to suspicion of abuse.

1. Review notes / speak to others/ social workers/ child protection liaison officer to obtain guidance on best way to proceed

2. Approach to child :

- Stay calm
- communicate in a way that is age appropriate
- provide a private and child friendly environment
- respond in a sensitive and caring manner
- provide support without being judgemental
- listen to what the child has to say
- use open ended questions
- do not probe for details or ask leading direct questions
- do not promise confidentiality
- minimise number of people examining or interviewing

Approach to parents/carers

- non-judgmental manner
- no leading questions
- empathise but not support abusive behaviour
- keep parents informed of child's medical conditions and needs

Documentation

Adult ADHD

You have been referred to see James by his GP for assessment of Adult ADHD. James has been having difficulty holding down his jobs. He has a 10 year old brother Clyde who has ADHD.

Outline how you would go about making a diagnosis of Adult ADHD.

1. Recognition that ADHD in adults presents differently to childhood

ADHD:Adult ADHD presents with inattention and organisational difficulties with reduced hyperactivity and impulsivity. It can also present primarily with depression, reckless behavior, agitation, anger, drug and alcohol issues misuse, gambling, thrill seeking behaviour.

2. Assessment needs to be carried out longitudinally and patient to be told that diagnosis may take several appointments and medication is not prescribed lightly.

3. ADHD in adults has a high co-morbidity and other Axis I disorders such as depression, bipolar disorder, Drug and alcohol misuse and organic issues should be ruled out before making a conclusive diagnosis of ADHD.

4. Interview James and assess for onset of symptoms. current Symptoms ; inattention, disorganization,, running late etc Impact of condition of psychosocial functioning. MSE for fidgetiness, pressured speech etc.

5. Drug and Alcohol history: Current use / Previous dependence history/ Treatment and motivation to abstain.

6. Neurological examination and organic screening to rule out

: developmental delay, thyroid dysfunction, Vit B12 and folate deficiency, MRI if neurological issues identified. Sleep disturbances (insomnia, OSA, REM sleep movement disorder), Pain. ECG

7. Personality assessment: Rule out antisocial personality disorder and childhood conduct disorder.

8. **Collateral information** from school, family to corroborate onset of symptoms: School reports. Impulsivity, inattention and hyperactivity, pervasive and across settings (home, school, playground etc.)

9. ADHD scale - ASRS (ADHD self-report scale / CADDRA/ DIVA).

Some symptoms you can add from the following list to show presentations in adults - add this in the current symptom domain (Please note you only need 5-6 of below - be mindful of time)

- Mind seems elsewhere, even in the absence of any obvious distraction
- Starts tasks, but quickly loses focus and is easily side-tracked
- Fails to finish tasks in the workplace
- Reporting unrelated thoughts
- Problems returning calls, paying bills, keeping appointments
- Difficulty in managing sequential tasks; difficulty in
- Keeping materials and belongings in order; messy, disorganised work
- Poor time management
- Tends to fail to meet deadlines
- Feeling restless
- Unable or uncomfortable being still for an extended time, such as in restaurants or meetings
- Might be perceived by others as being restless and difficult to keep up with
- Butts into conversations or activities, might start using other people's belongings without permission, might intrude into or take over what others are doing
- Failing to attend an appointment or turning up late or on the wrong day
- Losing their prescriptions and referrals
- Poor compliance with medication, especially for multiple daily doses
- Poor compliance with referrals and tests
- Failing to undertake routine preventive health checks such as Pap smears
- Presenting after having unprotected sex, possibly with a pregnancy or sexually transmitted disease scare
- University students requesting medical certificates to get extensions of assignments
- Reporting feeling overwhelmed or procrastinating a lot
- May be overly talkative or lose focus and ask you to repeat things
- Shifting repeatedly in seat or jiggling legs
- Drinking excessive quantities of caffeine or energy drinks or excessive smoking of cigarettes
- Smoking marijuana to slow their mind and get to sleep

- Poor sleep routine, e.g. staying up too late on work nights or often on screens
- Failing to pay their medical bill (if private billing)
- Leaving their belongings behind in the practice, losing their wallet or credit card and poor budgeting

James has had difficulties in the social and occupational aspects of his life. He had been using his brother's methylphenidate with good effect. He has lost jobs in the past due to drug use. What are the main issues and how will you address them?

1. Risk: Explain to James that misusing his brother's medication can be dangerous and is not advised. Inform James that you will need to inform family as there is a risk of harm to his brother and that medication will need to be restricted.

2. Medico-legal issues: The brother's GP need to be informed as there is a risk to child. Seek advice from M-L insurance to assess any additional medico-legal requirement.

3. Longitudinal assessment : Advise James that an assessment for ADHD is longitudinal and medication may not be prescribed at this appointment.

4. Rule out Substance use and antisocial personality disorder.

5. What is his motivation to seek treatment?

6. Potential for diversion and misuse of medication

7. Potential for accidental overdose if combined with other drugs

8. If treatment prescribed explain treatment contract: Script cannot be reissued if lost. Random drug tests. Obtain medication from same pharmacy. Medication will be ceased if any diversion or non-benefit noted.

9. Contact doctor shopping site to see if any other prescriptions are obtained.

You have been asked to give a talk to a group of GP's regarding Adult ADHD.

Outline what you would discuss. (In the exam they could also ask you the approach)

1. If approach asked then consider educator role :

Survey the audience. Is it GP's or Registrars as well?

Pre-existing knowledge or asking them what they would like covered in a survey.

Audio-visual presentations, didactic and interactive learning based on principles of adult learning e.g case studies

2. Scholar Role:

- ADHD presents atypically in adults. Inattention and concentration may persist in adulthood whilst hyperactivity and impulsivity reduces which makes diagnosis difficult.
- Strong family history may be present.
- Conduct disorder may be present in childhood
- ADHD is highly co-morbid and hence other disorders should be ruled out before conclusive diagnosis is made

3. Clinician role:

- Presenting symptoms may be : aggression, irritability, depression, recklessness, poor work performance, procrastination etc.
- Rule out Depression, bipolar disorder, psychosis and substance use.
- Treatment options : Medication and Psychological strategies
- Medication consists of stimulants and non-stimulants
- Side effects and evaluation (ECG, cardiology review)
- When to refer to psychiatrist? and Referral pathways. Resources available.

4. Professional role:

- Medico-legal obligations around stimulant prescribing (Schedule 8 restricted drug)
- Doctor-patient treatment contract

What are the issues in prescribing of medication in Adult ADHD

Question 2 What are the Issues in prescribing medication in Adult ADHD.

1. Pre-treatment

- **Diagnostic clarification. ADHD in adults is often comorbid and hence comorbidity should be treated optimally. Second opinion**

- Use ADHD questionnaires e.g ADHD Self-report scale
- CADDRA or DIVA questionnaire
- Collateral information and school reports are an important part of assessment
- Medical evaluation to rule out cardiac disease
- History of psychosis or substance use that may be relative contraindications or cautions for use of stimulants

Treatment issues :

- First line treatment is stimulant.
- Methylphenidate or dexamphetamine that come as immediate or extended release is evidence based
- Monitor for side effects - Stimulants can increase anxiety, lead to insomnia or agitation and loss of appetite
- Tics can occur and may need clonidine or guanfacine either as single agent or in combination
- Start low and go slow. Maximum doses 60 mg of Methylphenidate and Dexamphetamine. Lisdexamfetamine also available

Medicolegal aspects :

- Authority prescribing – Important to check local prescribing medicolegal regulations for example S8 authority
- Bupropion is evidence-based if non-stimulant is needed

Post-treatment:

- Monitor for risk of diversion / comorbid substance use
- Monitor weight and BP
- ECG 6 monthly as needed
- If prescription misuse is suspected – check with prescription shopping programs e.g. safe script

Depression in Pregnancy

You are a psychiatric registrar working in a Community Mental Health Centre. You are asked to review Justine, a 32 year old female, recently married with no children, working full time as a medical secretary. Justine has recently told the practice nurse (at her local medical centre) that she would like to become pregnant.

She has been hospitalised twice in the past, including an episode after a suicide attempt five years ago. She has been well for two years. She is prescribed paroxetine, 40 mg daily, and has no significant side effects. Justine has been referred by her General Practitioner, having previously been treated at the CMHT for recurrent major depression.

Outline your approach to the interview with Justine .

1. Process of interview with Justine: See Justine on her own to explore wishes and concerns; invite partner to the interview to elicit longitudinal and corroborative information.

2. Importance of planning the pregnancy - When does the patient want to start planning? Is the patient currently on contraception or is the partner using contraception? (If the question asked about what advice you would give - then it is important to tell the patient about planning the pregnancy. Discuss contraception and then go on to advise the risk-benefit analysis)

3. Review Justine's past history: Her mental state when not taking medication, response to other antidepressant medication, symptoms of hypomania or mania. The severity of illness, Risk of harm to self or others. Current capacity to make decisions.

4. Assess Justine's current mental state: Current mood (euthymic, depressed or hypomanic), judgement, insight, risk assessment. Current risk.

5. Discussion of treatment of depression in pregnancy: Risks of treatment with antidepressants in pregnancy and breastfeeding, risks of no treatment, provision of written materials, non-pharmacological treatments, assessing Justine's and her partner's attitudes to treatment.

- Paroxetine is associated with PPHN in 3rd-trimester exposure.
- Neonatal withdrawal syndrome

- Safe antidepressants - sertraline / escitalopram
- Risk of withdrawal symptoms with reduction and cessation of paroxetine. Gradual tapering is required. This may take up 3-6 months in some patients.
- Risk of depression in pregnancy - neglect, suicide risk, attachment and bonding impacts

6. Liaison issues : explain that a pregnancy will require liaison with obstetrician/midwife/ high risk pregnancy service. Role of GP, Perinatal psychiatry service.

7. Follow up arrangements: Arrange follow up appointment within a short time frame to discuss their concerns/ decision. Advise contraception until decision is made.

Note - In the exam they can ask about IVF - patient wants to go ahead with IVF. outline key issues - similar aspects apply - but capacity to make decisions and parenting capacity aspects will be important as part of whole process.

Also, it is especially important to focus on past history and current mental state. Current level of unwellness will be important

Furthermore - IVF involves hormone Rx - impact on mental illness - worsening monitoring/stress of IVF and relapse.

Collaborative approach with IVF specialist / psychological support (specialised psychologists in this area)

Other components remain the same - Risk-benefit analysis. Risk-benefit analysis of IVF as above and Risk-benefit analysis of the psychiatric medication and illness.

Management of Pain and Addiction on Ward

You are working as the junior consultant in a large hospital. Tony aged 23 years was involved in a motorcycle accident in which he sustained multiple internal injuries and an unstable neck fracture. Tony has surgery to stabilise his cervical spine and is currently immobilised in a halo traction , has a chest drain and cannot be moved.

Two days after surgery, the spinal injury team contacts you expressing concern at Tony's excessive demands for opiates. When you speak to Tony he tells you that he is in agony and can't sleep because of back pain. He looks anxious and low in mood but denies anxiety and depressive symptoms.

Tony is demanding to smoke cannabis and cigarettes and says he cannot cope with hospital without them. He has threatened to sign himself out of hospital. He is verbally aggressive towards staff especially when they try to discuss the hospital smoke free policy with him. the staff would like your help in managing Tony.

You registrar says the diagnosis is Antisocial Personality Disorder

Describe (List and elaborate) the major diagnostic possibilities you would discuss with the registrar.

(Exam answer) : As per marking scheme:

Withdrawal not mentioned (GAP) especially after two days worsening of behaviour. May be subsumed under substance use disorder.

1. Personality Dysfunction / Maladaptive coping strategies : affect dysregulation / low frustration tolerance/impulsivity e.g he has threatened to sign himself out of the hospital. He is verbally aggressive towards staff.

2. **Inadequate pain management** e.g he tells you that he is in agony and can't sleep due to back pain.

3. **Substance abuse/dependence disorder** e.g he is verbally aggressive towards staff especially when they try to discuss hospital smoke free policy with him. Demanding to smoke cannabis and cigarettes and that he cannot cope without them.

4. **Acute stress disorder:** he looks anxious and low in mood, but denies anxiety or depressive symptoms.

5. **Delirium** e.g acute severe injury two days post-surgery

6. **No diagnosis (deferred)** / Interactional difficulties between patient and staff.

You are working as the junior consultant in a large hospital. Tony aged 23 years was involved in a motorcycle accident in which he sustained multiple internal injuries and an unstable neck fracture. Tony has surgery to stabilise his cervical spine and is currently immobilised in a halo traction , has a chest drain and cannot be moved.

Two days after surgery, the spinal injury team contacts you expressing concern at Tony's excessive demands for opiates. When you speak to Tony he tells you that he is in agony and can't sleep because of back pain. He looks anxious and low in mood but denies anxiety and depressive symptoms.

Tony is demanding to smoke cannabis and cigarettes and says he cannot cope with hospital without them. He has threatened to sign himself out of hospital. He is verbally aggressive towards staff especially when they try to discuss the hospital smoke free policy with him. the staff would like your help in managing Tony.

You registrar says the diagnosis is Antisocial Personality Disorder

Tony gives a history of heavy use of cigarettes (30 per day) and daily cannabis use (2-3 joints a day)

Discuss (list and debate) what strategies you would advise your registrar might help to manage without cigarettes and cannabis.

Exam answer as per marking scheme:

1. **Psychoeducation** : Reasons for staying in hospital, likely length of stay, likely withdrawal, ways he can be helped with these.

2. Behavioral techniques: Relaxation, distraction, positive self talk, scheduling positive activities (visit from friends and family, walks etc.)

3. Pharmacotherapy : Ensure adequate NRT patches/gum/inhaler, Bupropion and varenicline, SSRI if concomitant anxiety and depression, Short term BZD's for acute anxiety.

4. Staff management strategies : Positive encouragement from staff. Placing in room with young adults. Avoid confrontation whilst quietly setting boundaries. Education for staff on how to build a collaborative care relationship with patient.

Additional Points :

1. Therapeutic alliance, non-judgmental attitude acknowledging that it is difficult to stop suddenly.

2. Explain policy of ward and that treatment will proceed in his best interests while adhering to policies.

3. Be aware for potential for escalation, acting out and educate staff.

4. Psychoeducation for staff so they adopt a non-judgemental attitude.

5. Motivational interviewing techniques : positive affirmation, validation to try to consider moving patient to action phase.

6. Involve drug and alcohol services

You are working as the junior consultant in a large hospital. Tony aged 23 years was involved in a motorcycle accident in which he sustained multiple internal injuries and an unstable neck fracture. Tony has surgery to stabilise his cervical spine and is currently immobilised in a halo traction , has a chest drain and cannot be moved.

Two days after surgery, the spinal injury team contacts you expressing concern at Tony's excessive demands for opiates. When you speak to Tony he tells you that he is in agony and can't sleep because of back pain. He looks anxious and low in mood but denies anxiety and depressive symptoms.

Tony is demanding to smoke cannabis and cigarettes and says he cannot cope with hospital without them. He has threatened to sign himself out of hospital. He is verbally aggressive towards staff especially when they try to discuss the hospital smoke free policy with him. The staff would like your help in managing Tony.

You registrar says the diagnosis is Antisocial Personality Disorder

Tony gives a history of heavy use of cigarettes and daily cannabis use (2-3 joints per day)

Outline (List and justify) the issues you would raise with your registrar in managing Tony's pain.

1. Assessment of pain:

Discussion with surgical team re extent of surgery, post op complications, any possible organic contributors to pain.

Assessment of pain symptoms, pain charts, vital signs, medication charts to date.

2. Psychological factors: Explore and address any psychological contributors to pain such as anxiety, fear and misunderstandings re possible outcomes.

3. Pharmacotherapy: Regular rather than as required analgesic medication, appropriate pharmacotherapy for co-morbid psychiatric conditions, do not withhold analgesia on basis of previous drug use.

4. Non-pharmacological pain management: Distraction, OT

5. Staff liaison and education: Address staff issues such as hostile countertransference , educate staff that pain is genuine and needs pain medication.

6. Involvement of Pain service or Drug and alcohol service.

Indigenous Psychiatry

Mr. Davies is a 60 year old Aboriginal man with chronic paranoid schizophrenia. He lives with his son, daughter-in-law and 2 grandchildren (3 and 5 years old). He is on a flupenthixol depot 20 mg IM 2 weekly and has been managed in the community. In the past when unwell, he has responded to an increased dose in his medications. He has a past history of verbal aggression and paranoia toward the neighbours. Recently, a member of the family passed away and Mr. Davies' psychotic symptoms worsened - he started expressing more paranoid persecutory ideation regarding his neighbours trying to poison him. He thinks that the only way to solve this is to smoke them by burning their houses. He also thinks that his 4 year-old grandson, Tyler, is trying to poison him. Mr Davies has lost weight as a result of this.

List the risks inherent in the scenario. (Please justify if outline is asked with evidence from vignette)

1. Risk of harm to child : He has persecutory delusions against the 4 year old.
2. Risk of harm to self e.g. suicide or self harm : Grief leading to depression with psychosis
3. Risk of harm to neighbours
3. Medical risk: Self neglect, poor appetite and dehydration. (Loss of weight)
4. Risk of re-feeding syndrome during treatment
5. Risk to reputation with subsequent stigma
6. Risk of disengagement with community services and risk of worsening mental state
7. Risk of misdiagnosis: Cultural elements may be present secondary to grief.

Mr. Davies' son thinks that you need to increase the dose of Mr. Davies depot as this worked in the past. (The son wants the father to be managed at home.

He believes he can convince his father to take some oral medications (Olanzapine). You agree to the plan. However, the Aboriginal liaison worker thinks that this is a normal cultural response to a death in the family and not a relapse of schizophrenia. He feels patient should not be managed with medication, that he should be managed with traditional healing instead and wants to involve a traditional healer.

The outline you approach the situation:

Patient first; Listen and understand reasons for the decision; then psychoeducation

1. Assess Mr Davies current mental state and capacity to make decisions (**autonomy**). If Mr Davies lacks capacity and there is a risk of harm to self or others then the family should be informed that a mental health act assessment will need to be carried out. (**Clinician and Professional Role**)
2. If Mr Davies has capacity then ask for his wishes with regards to treatment. Arrange a meeting with Mr Davies, his son and traditional healer to clarify concerns. (**Collaborator role**). **Understand the reasons why they want a traditional healer involved.**
3. Approach the situation in a non-judgemental manner and aim to establish a therapeutic alliance. Recognise the importance of cultural practices but at the same time provide psychoeducation about the importance of treatment and risks associated with illness. (**Collaborator and educator role**)
4. Explain that both treatments can co-exist. Clarify if traditional healing practices would mean that antipsychotic medication would need to be stopped. (**Clinician role**). **Any interactions with psychiatric medications?**
5. Explain the risks of non-treatment of schizophrenia. Explain the risk and benefits of medications. **Explain role of mental health clinicians**
6. Involvement of community elders as needed to build trust

Mr. Davies' son approaches you as his wife wants to leave the family home as she feels threatened by Mr. Davies. He has been looking suspiciously at the grandchildren. Mr Davies' son feels torn between his loyalties to his wife and father. He asks for your advice.

Outline your approach.

1. Arrange a meeting and listen to Son's and wife's concerns: Take a non-judgmental approach and establish a therapeutic alliance. Rule out the risk to children. If risk to children is present immediate assessment and admission may need to be considered. Legislative requirements (e.g MHA assessment)

2. Explain that priority is to treat Mr Davies' illness. Explain to the family that behaviour is driven by mental illness and that adequate treatment of mental illness could allow Mr Davies to return home safely.

Short term Separation (e.g admission) may be necessary as part of treatment due to risks involved. Admission under the Mental health act may be required if Mr Davies' lacks capacity and risk is significant.

3. Provide psychoeducation about the illness and that the situation may change once the illness is treated.

4. Involve Indigenous liaison worker

5. Consider overnight leave to assess the situation at home

5. Arrange to follow up meetings to assess the situation at regular intervals.

CTO and Ethics

26 year male with Hx of Schizoaffective disorder and Meth dependence.

Patient is released from prison 2 weeks ago on depot clopixon and olanzapine. Currently on enforced treatment MHA (not forensic licence). He tells you that he is drug free as he is required to do random UDS for the next 6 months part of his bail condition.

He came to see you with his sister who is adamant that he should not be on a treatment order, and that he has rights to choose his own treatment.

Q1. List and explain the possible ethical issues that arises from this situation.

1. Autonomy and informed decision making (capacity to make an informed decision)

- What is patient's insight
- Patient's attitude to treatment
- Does patient consent to CTO / Enforced Treatment Order
- Does patient have capacity to consent to order or voluntary treatment
- Power of attorney in medical decision making

2. Beneficence:

Is CTO in the best interest of the patient?

Justification for CTO:

- Patient has a treatable mental illness that has to be treated
- Medication and treatment proposed is in best interest of patient as reduces risk of relapse and treats symptoms

- The patient does not want to continue to take medication, which is likely to help improve symptoms and reduce the risk of relapse.
- Previous treatment in hospital has not been sufficiently effective to lead to remission or improvement
- Discharge from hospital has resulted in relapse.
- Patient understands the role of a Community Treatment Order and accepts the conditions of the Community Treatment Order.
- Community Treatment Order is the least restrictive option for the patient.
- CTO or enforced order is the least restrictive option for the patient
- Patient lacks insight and is non adherent with treatment (current and past)
- Patient chooses a CTO / Enforced order

3.Non-Maleficence:

- CTO can have a negative impact on therapeutic alliance
- A Community Treatment Order may undermine the autonomy of the patient, which is a key tenant of the recovery model.
- Community Treatment Orders can disrupt the therapeutic alliance, which is associated with good outcomes in mental illness.
- Despite a Community Treatment Order, patients may relapse.

4.Confidentiality and Privacy

- Family members involvement is important in treatment.
- Autonomy and mental health act takes precedence over family wishes.
- Family wishes are conveyed to tribunal

Q2. 26 you with Hx of Schizoaffective disorder and Meth dependence. Released from prison 2 weeks ago.on depot clopixol and olanzapine. Currently on enforced treatment MHA (not forensic licence). He tells you that he is drug free as he is required to do random UDS for the next 6 months part of his bail condition.

He tells you that he will not accept any treatment unless he is on a treatment order. Sister confirms that they will be relocating to rural area soon. He will be referred to the CMH in the region for follow up.

What are the possible risks involved in terms of historical, social and clinical aspects in this man with regards to relapse?

1.Historical factors associated with increased risk of relapse

- History of severe mental illness

- History of substance misuse
- Forensic involvement
- Depot medication may indicate non-adherence
- Depot medication (clopixol depot) indicative of relapse in the context of non-compliance
- Previous supervision failure

Other factors include – lack of employment / relationships / personality factors / antisocial behaviour / violence / traumatic experiences

2. Clinical factors that increase risk of relapse

- Active Symptoms of SMI
- Ongoing substance use
- Supervision and engagement with services
- Insight into illness

3. Social factors that increase risk of relapse:

- Rural location may make follow up difficult
- Lack of personal support to detect early relapse signs
- Family member with negative attitudes to treatment may prevent adequate treatment being implemented or detect relapse
- Living situation – access to drugs
- Availability of resources in rural CMHT.

4. Other factors : coping strategies / stress

Q2. List and explain principles of informed consent.

- Capacity
- Adequate info
- Reasonable opp to make decision
- Given consent freely
- Not withdrawn
- All people are presumed to have capacity to give informed consent to treatment or medical treatment regardless of their age or legal status under the Mental Health Act

1.Capacity:

- This means that everyone must be presumed to have capacity to make decisions about their treatment or medical treatment, regardless of their age (e.g. young people or older persons) or whether they are a patient under the Mental Health Act.

2. Adequate Information:

- Have been given adequate information to enable the person to make an informed decision
- They have been given an explanation of the proposed treatment or medical treatment, including the purpose, type, method and likely duration of the treatment or medical treatment
- They have been given an explanation of the advantages and disadvantages of the treatment or medical treatment including information about the associated discomforts, risks and common or expected side effects of the treatment or medical treatment
- They have been given an explanation of any beneficial alternative treatments that are reasonably available, including any information about the advantages and disadvantages of these alternatives
- They have received answers to any relevant questions that the person has asked and any other relevant information that is likely to influence the person's decision
- They have been given the relevant statement of rights and had that statement explained to them in a manner that the person is most likely to understand.

3. Reasonable Opportunity:

- Time
- Opportunity to discuss with doctor
- Reasonable support
- Opportunity **to seek advice or assistance to make decision**

4. Freedom from undue influence

- Consent given freely; no coercion from any other individual including medical practitioner

5. Not withdrawn Consent

- Verbally or in writing
- Behaviour indicating lack of consent